2

Article 12 – Context and Background

1 INTRODUCTION

This chapter introduces the main interpretive arguments surrounding Article 12. It does this by first contextualising them against key policy and legal concepts in disability and decision-making by, with and for adults with cognitive disability, as they have developed over the twentieth and twenty-first centuries. Chapter 1 described how disability human rights development culminated in the CRPD, tracking a journey from paternalism to autonomy, from invisibility to visibility and from discrimination to equality. It also introduced the idea of autonomy – demanding the ability to reason, to be independent and to be free from interference from the State – as the foundation of personhood. The development of relevant policy and legal concepts in disability is explained in this chapter in terms of how it reflects that evolutionary journey to increasing levels of autonomy, recognition of personhood, and equality. However, it also anticipates a radical disruption of that dominant narrative (to be developed in subsequent chapters) and examines the complexities around those concepts, as revealed by the case of cognitive disability.

Section 2 describes the different 'models' of disability – the charity, medical and social models – as developed and widely used by disability advocates and commentators to explain how various policy and legal responses to disability have developed and functioned. Section 3 describes how the development of the CRPD was hailed as articulating a social model of disability, whereby disability is framed as a product of a prejudiced, ableist society. It further explains how a strict social model of disability has since been subject to wide critique. It describes how the interpretive arguments made in this book are explained by referencing a social model of disability that recognises the residual impacts of impairment.

Section 3 goes on to describe the different policy and legal responses to decision-making by, with and for adults with cognitive disability that have aimed to improve adults' autonomy, increasingly recognise their value as persons and eliminate discrimination. The era of mass institutionalisation was followed by the development and implementation of personal guardianship regimes. Within those regimes,

capacity tests evolved from status and outcome tests to functional tests. Decision-making principles have begun to evolve from paternalistic 'best interests' tests to acknowledging the will and preferences of adults and recognising their individual autonomy. Since the 1990s, supported decision-making practices and legislation have developed, which aim to further increase the autonomy of adults with cognitive disability and reduce or eliminate discrimination on the basis of capacity.

Section 4 provides first an overview of the major arguments made during the drafting of Article 12, and afterwards, for interpreting Article 12 as requiring the abolition of decision-making by substitutes. This is followed by an overview of those arguments that interpret Article 12 as allowing for or requiring decision-making by substitutes as a last resort. Section 4 describes how the interpretive contention over Article 12 led to the publication of *General Comment No 1* in 2014, which prescribes an interpretation of Article 12 as requiring the abolition of decision-making by substitutes and guardians in all cases, and their replacement with practices of supported decision-making.[1] There are various arguments made in support of this interpretation, all based on the premise that the right to legal capacity equates with a right to autonomy, and that denial of legal capacity therefore means denial of personhood.[2] It follows that to deny recognition of legal capacity to people on the basis of their disability will always constitute unlawful discrimination.[3] The contrary arguments are premised, to a greater or lesser extent, on an understanding that hard reality dictates decision-making by substitutes may not be a choice but an inevitability,[4] and respect for human rights demands that it be recognised as such and made transparent and accountable.[5]

[1] *Committee on the Rights of Persons with Disabilities, General Comment No 1 Article 12: Equal Recognition before the Law UN Doc CRPD/C/GC/1 (19 May 2014)* ('*General Comment No 1*') [3], [7], [26].

[2] 'The right to equal recognition before the law implies that legal capacity is a universal attribute inherent in all persons by virtue of their humanity …': *General Comment No 1* [8], [26], [29]; 'It has been found that denial of legal capacity to any individual or group has also meant negation of both the right to personhood and the capacity to act': International Disability Alliance, *Legal Opinion on Article 12 of the CRPD* (disability-studies.leeds.ac.uk/wp-content/uploads/sites/40/library/legal-opinion-LegalOpinion-Art12-FINAL.pdf008) 38–52; Nandini Devi, 'Supported Decision-Making and Personal Autonomy for Persons with Intellectual Disabilities: Article 12 of the UN Convention on the Rights of Persons with Disabilities' (2013) 41(4) *The Journal of Law, Medicine & Ethics* 792; Jillian Craigie et al, 'Legal Capacity, Mental Capacity and Supported Decision-Making: Report from a Panel Event' (2019) 62 *International Journal of Law and Psychiatry* 160, 161; Glen, Kristin Booth, 'Introducing a New Human Right: Learning from Others, Bringing Legal Capacity Home' (2018) 49(3) *Columbia Human Rights Law Review* 1, 20.

[3] *General Comment No 1* [7], [15]; Anna Arstein-Kerslake and Eilionóir Flynn, 'The General Comment on Article 12 of the Convention on the Rights of Persons with Disabilities: A Roadmap for Equality before the Law' (2016) 20(4) *The International Journal of Human Rights* 471, 478.

[4] Wayne Martin et al, *Achieving CRPD Compliance: Is the Mental Capacity Act of England and Wales Compatible with the UN Convention on the Rights of Persons with Disabilities? If Not, What Next?* (Essex Autonomy Project, 2014) 23; Craigie et al (n 2).

[5] Matthew Burch, 'Autonomy, Respect, and the Rights of Persons with Disabilities in Crisis' (2017) 34(3) *Journal of Applied Philosophy* 389, 390, 397; Malcolm Parker, 'Getting the Balance Right: Conceptual Considerations Concerning Legal Capacity and Supported Decision-Making'

2 DIFFERENT 'MODELS' OF DISABILITY

From the late twentieth century, the commentary on disability has described what are widely acknowledged as three dominant models of disability – the charity, medical and social models – which underlie, or have driven, the development of disability policy and law.[6] These models are described in the following sections as they have been widely presented in the disability literature – that is, as reflecting evolving social attitudes to people with disability along a continuum from paternalism to respect for autonomy, from exclusion to inclusion and from discrimination to equality. The section goes on to describe wide criticisms of the social model and how the arguments in this book are premised on a social model of disability that recognises the residual impacts of impairment.

2.1 *Charity Model*

Commentators relate how, before the late twentieth century, the only morally defensible attitude of the State and the wider community towards people with disability was to view them as objects of pity and charity.[7] People with disability were also encouraged to perceive themselves in this limited role, internalising community prejudices. The Article 12 literature and the commentary surrounding the CRPD continually reiterate that Article 12 and the CRPD more broadly represent a move away from the 'charity model' of disability to either a 'social model' or 'human rights model' of disability (see further sections).[8]

This perception, of people with disabilities as objects of charity, is reflected in the work of the legal philosopher John Rawls, whose broader influence on the liberal theory of autonomy is described in Chapter 4.[9] Rawls, in his book *A Theory of Justice*, drew upon Kantian theories to develop his theory of justice as fairness.[10] For Rawls, society is constituted by individuals who each agree to enter into a 'social contract' with each other so that the State operates on a foundation of common rules and understandings for the mutual advantage of individuals who pursue their own conceptions of 'the good': 'A person's good is determined by what is for him

(2016) 13(3) *Journal of Bioethical Inquiry* 381, 389; Kjersti Skarstad, 'Ensuring Human Rights for Persons with Intellectual Disabilities?' (2018) 22(6) *International Journal of Human Rights* 774, 777; Silvana Galderisi, 'The UN Convention on the Rights of Persons with Disabilities: Great Opportunities and Dangerous Interpretations' (2019) 18(1) *World Psychiatry* 47, 47.

[6] Shane Clifton, *Hierarchies of Power: Disability Theories and Models and Their Implications for Violence against, and Abuse, Neglect, and Exploitation of, People with Disability* (Royal Commission into Violence, Abuse, Neglect and Exploitation of People with Disability, 2020) 4–13.

[7] Australian Law Reform Commission, *Equality, Capacity and Disability in Commonwealth Laws: A Final Report* (2014) 4–5.

[8] For the 'social model', see Section 2.3, and for the 'human rights model', see Section 2.4.4.

[9] See Chapter 4, Section 2.

[10] John Rawls, *A Theory of Justice* (Harvard University Press, 1971).

the most rational long-term plan'.[11] This 'contract' model or metaphor relies on the individual having reasoning abilities to a requisite 'level', so that citizenship is only bestowed on those who are 'beyond a certain age and possessed of the requisite intellectual capacity'.[12]

The social contract can include some rules for redistribution of resources from the more advantaged to the disadvantaged when this redistribution is for the benefit of society overall. However, it does not provide for redistribution to those who are deemed unable to contract and are therefore ineligible for the rights of citizenship.[13] For Rawls, citizens have liberties and are the subjects of rights, but those denied citizenship because of their cognitive impairment are the objects of paternalism.[14] So, any distribution of resources to those 'non-citizens' is entirely discretionary. This positions those with cognitive disability as objects of pity, not the subjects of rights, and it imposes the values of the donor on the charitable objects, who are assumed to have no ability to know what is good for them.[15]

Disability advocates and scholars clearly reject the '…old worldview (objectification of persons with disabilities)',[16] which defined any social or economic distribution to people with cognitive disability as an act of 'charity' for those outside the citizenship State.[17] The so-called 'charity model' of disability, although still lingering (or some may say pervasive) in various community responses and attitudes,[18] is widely rejected in disability commentary and advocacy.[19] A charity model denies autonomy and personhood to adults with disability and fails to position them as holders of human rights.[20]

[11] Ibid 25.
[12] Ibid 46.
[13] Ibid 87.
[14] Ibid 250.
[15] Ibid 249.
[16] Gerard Quinn, 'Resisting the "Temptation of Elegance": Can the Convention on the Rights of Persons with Disabilities Socialise States to Right Behaviour?' in Oddný Mjöll Arnadóttir and Gerard Quinn (eds), *The UN Convention on the Rights of Persons with Disabilities: European and Scandinavian Perspectives* (Nijhoff, 2009) 215, 216.
[17] Barbara Arneil and Nancy J Hirschmann, 'Introduction' in Barbara Arneil and Nancy J Hirschmann (eds), *Disability and Political Theory* (Cambridge University Press, 2016) 1, 7.
[18] Clifton (n 6) 6.
[19] Ibid 4–6; John Chesterman, 'The Future of Adult Guardianship in Australia' (2013) 66(1) *Australian Social Work* 26, 30; Gerard Quinn and Anna Arstein-Kerslake, 'Restoring the "Human" in "Human Rights": Personhood and Doctrinal Innovation in the UN Disability Convention' in Costas Douzinas and C A Gearty (eds), *Cambridge Companion to Human Rights Law* (Cambridge University Press, 2012) 36; Tom Shakespeare, *Disability: The Basics* (Routledge, 2018) 15; Rosemary Kayess and Philip French, 'Out of Darkness into Light? Introducing the Convention on the Rights of Persons with Disabilities' (2008) 8(1) *Human Rights Law Review* 1, 3.
[20] Often the commentary will equate the 'charity' model with a 'welfare' model of disability, so that the two terms are elided or used interchangeably. See Arstein-Kerslake and Flynn (n 3) 471. This elision I suggest reflects the experiences of people dealing with state welfare agencies that administer their services in ways that objectify and humiliate recipients. See Shakespeare,

2.2 *Medical Model*

The pre-eminence of the charity model was to some extent displaced by the emergence of the 'medical model' of disability. The medical model is described as constructing disability purely or foremostly as a condition to be remedied or rectified (or not) by intervention – whether medical or otherwise.[21] There were and are good reasons for encouraging medical and health responses to impairments, and Arneil describes how the medical model was associated with rehabilitation programmes for injured World War One veterans. However, Arneil also describes how this focus led to rehabilitation being an 'overarching paradigm governing disability in the twentieth century' to the exclusion of other paradigms. To this extent, there became a dominant societal assumption behind the medical model that those who were unable to be fully rehabilitated were somehow lesser beings.[22]

This so-called medical model has been described as constructing disability as being essentially caused by a fault or defect within the individual.[23] As such, it was the individual's responsibility to either remedy the disability by seeking intervention or, failing that, the disability was to be borne by the individual through personal adaptation or stoicism.[24] In a society where people with disability were largely socially and politically excluded, the idea that disability should, if possible, be rectified or remedied by medical science engaged the corollary implication that people with untreatable conditions or impairments were 'defective' or less than normal.[25] As Erevelles has described it, the medical model served to position disability 'as a

Disability: The Basics (n 19) 76; Clifton (n 6) 6–7; Jennifer Nedelsky, *Law's Relations: A Relational Theory of Self, Autonomy, and the Law* (Oxford University Press, 2012) 66. However, I argue that equating the words 'charity' and 'welfare' in this context risks perpetuating the stigmatisation of social welfare claimants and the services to which they are entitled as a matter of human rights (see especially Chapter 3, and Section 4.2.1 of this chapter). For this reason, I exclusively use the term 'charity model'.

[21] Clifton (n 6) 7–9; Michael Ashley Stein, 'Disability Human Rights' (2007) 95(1) *California Law Review* 75, 87–8.

[22] Arneil and Hirschmann (n 17) 30.

[23] Stephen R Smith, 'Social Justice and Disability: Competing Interpretations of the Medical and Social Models' in Kristjana Kristiansen, Simo Vehmas and Tom Shakespeare (eds), *Arguing about Disability: Philosophical Perspectives* (Routledge, 2008) 15, 15.

[24] Martha Rioux, 'On Second Thoughts: Constructing Knowledge, Law, Disability and Inequality' in Stanley S Herr and Lawrence O Gostin (eds), *The Human Rights of Persons with Intellectual Disabilities: Different but Equal* (Oxford University Press, 2003) 287, 291.

[25] 'The medical (or biomedical) model considers disability to be a problem with the individual that is directly caused by a disease, an injury, or some other health condition and requires medical care in the form of treatment and rehabilitation': Willene Holness, 'Equal Recognition and Legal Capacity for Persons with Disabilities: Incorporating the Principle of Proportionality' (2014) 30(2) *South African Journal on Human Rights* 313, 316; Stein (n 21); Jan Wallcraft and Kim Hopper, 'The Capabilities Approach and the Social Model of Mental Health' in Helen Spandler, Jill Anderson and Bob Sapey (eds), *Madness, Distress and the Politics of Disablement* (Policy Press at the University of Bristol, 2015) 83, 84; Shih-Ning Then, 'Evolution and Innovation in Guardianship Laws: Assisted Decision-Making' (2013) 35(1) *Sydney Law Review* 133, 136–7.

pathological abnormality [and] has been used to support the exclusionary, segregationist, and exploitative practices of an ableist society'[26] with 'ableism' defined elsewhere as 'modalities, mechanisms, and techniques of marginalisation and oppression specifically directed at disabled persons due to their disabilities'.[27] Ableism has the potential to make society view people with disability as merely 'potential beings', as not fully formed, and thus fail to deny them full personhood.[28] The dominance of the medical model, as described above, to the exclusion of alternative perspectives has been widely criticised as inconsistent with a human rights approach to disability, which demands social inclusion.

2.3 *Social Model*

With the increasing emergence of human rights movements, disability advocates began to reject the old constructs of disability in favour of what is now known widely as the 'social model' of disability.[29] The social model was initially developed by

[26] Nirmala Erevelles, *Disability and Difference in Global Contexts: Enabling a Transformative Body Politic* (Macmillan, 2011) 151. Some philosophers have argued (contentiously) that the descriptors 'abnormal', 'atypical' or 'pathological' in relation to biological functioning can be descriptive without being normative; that is, there is an argument that they can be value neutral, without implying *inferiority* of functioning. See Christopher Boorse, 'On the Distinction between Disease and Illness' (1975) 5(1) *Philosophy & Public Affairs* 49, 555; Christopher Boorse, 'Health as a Theoretical Concept' (1977) 44(4) *Philosophy of Science* 542; Christopher Boorse, 'Disability and Medical Theory' in D Christopher Ralston and Justin Ho (eds), *Philosophical Reflections on Disability* (Springer, 2010) 55. For arguments against, see Ron Amundson, 'Against Normal Function' (2000) 31(1) *Studies in the History and Philosophy of Biology and Biomedical Science* 33; Elselijn Kingma, 'What Is It To Be Healthy? (Boorse's Biostatistical Theory)' (2007) 67(2) *Analysis* 128. Of relevance here is that even if we accept that some functioning is atypical in a descriptive sense, the *dominance* of the medical model meant that certain impairments have been *socially* constructed as 'abnormal' in a way that stigmatises, marginalises and devalues the person with disability.

[27] Lucas G Pinheiro, 'The Ableist Contract: Intellectual Disability and the Limits of Justice in Kant's Political Thought' in Barbara Arneil and Nancy J Hirschmann (eds), *Disability and Political Theory* (Cambridge University Press, 2016) 43, 46.

[28] Licia Carlson defines cognitive abelism as 'a prejudice or attitude of bias in favour of the interests of individuals who possess certain cognitive abilities … against those who are believed not to actually or potentially possess them.': Licia Carlson, 'Cognitive Ableism and Disability Studies: Feminist Reflections on the History of Mental Retardation' (2001) 16(4) *Hypatia* 124, 140.

[29] Anna Arstein-Kerslake, *Restoring Voice to People with Cognitive Disabilities: Realizing the Right to Equal Recognition before the Law* (Cambridge University Press, 2017) 12; Rannveig Traustadóttir, 'Disability Studies, the Social Model and Legal Development' in Oddný Mjöll Arnadóttir and Gerard Quinn (eds), *The UN Convention on the Rights of Persons with Disabilities: European and Scandinavian Perspectives* (Nijhoff, 2009) 3, 3–4; Oddný Mjöll Arnadóttir, 'A Future of Multidimensional Disadvantage Equality' in Oddný Mjöll Arnadóttir and Gerard Quinn (eds), *The UN Convention on the Rights of Persons with Disabilities: European and Scandinavian Perspectives* (Nijhoff, 2009) 41, 59; Theresia Degener, 'A New Human Rights Model of Disability' in Valentina Della Fina, Rachele Cera and Giuseppe Palmisano (eds), *The United Nations Convention on the Rights of Persons with Disabilities: A Commentary* (Springer International Publishing, 2017) 41, 41–4.

British scholars whose Marxist leanings led them to focus on disadvantage and economic inequality in the external, social environment as defining the notion of disability.[30] This model distinguishes between 'impairment' and 'disability'. Physical or cognitive disability is not framed as defining the individual or his or her identity but is understood as a product and function of the social and physical environment in which the individual is situated.[31] This model emphasises that it is the social and physical environment that places barriers to functioning on those with disability or fails to accommodate their impairments.

Because the emphasis is on barriers to functioning, a social model does not construct disability as an immutable status imposed on the person or as something that needs to be addressed by 'fixing' the person. Instead, the social model recognises the need for the State to provide supports and 'reasonable accommodation' to promote equality of opportunity for those with disability.[32]

The wide acceptance of this social model of disability led to the call for the development of the CRPD, and the CRPD is touted by many as being the first human rights instrument that fully incorporates the social model of disability.[33] The CRPD is said to embrace the social model by rejecting both the charity model's positioning of people with disability as objects of pity and the medical model's framing of disability as personal affliction or tragedy, so as to reposition people with disability as rights bearers.[34] As many commentators have written, the social model has served as an extremely effective political and advocacy tool in providing visibility to those with disability, in giving people with disability the confidence to make claims against governments, and in holding governments and the community responsible for addressing the needs of those with impairments.[35]

[30] Shakespeare, *Disability: The Basics* (n 19) 12.

[31] Then (n 25) 137, 138.

[32] 'Reasonable accommodation means necessary and appropriate modification and adjustments not imposing a disproportionate or undue burden, where needed in a particular case, to ensure to persons with disabilities the enjoyment or exercise on an equal basis with others of all human rights and fundamental freedoms': *CRPD* Article 2.; see further in Chapter 6, Section 2.6.1.

[33] Anna Lawson and Mark Priestley, 'The Social Model of Disability: Questions for Law and Legal Scholarship?' in Peter Blanck and Eilionoir Flynn (eds), *Routledge Handbook of Disability Law and Human Rights* (Routledge, 2016) 3; 'The social model is embedded within the CRPD'. Arstein-Kerslake, *Restoring Voice to People with Cognitive Disabilities* (n 29) 72; 'The salience of support is a concrete expressoin of the social, interactive model of disability that animates the Convention …': Robert D Dinerstein, 'Implementing Legal Capacity under Article 12 of the UN Convention on the Rights of Persons with Disabilities: The Difficult Road from Guardianship to Supported Decision-Making' (2012) 19(2) *Human Rights Brief* 8, 9; Australian Law Reform Commission ('ALRC'), *Equality, Capacity and Disability in Commonwealth Laws: A Final Report* (2014) 38; Michael Perlin, 'Understanding the Intersection between International Human Rights and Mental Disability law: The Role of Dignity' in Bruce Arrigo and Heather Bersot (eds), *Routledge Handbook of International Crime and Justice Studies* (Routledge, 2013) 191, 195.

[34] ALRC (n 33) 44.

[35] Kayess and French (n 19) 7; Shakespeare, *Disability Rights and Wrongs* (Routledge, 2006) 18; William J Penson, 'Unsettling Impairment: Mental Health and the Social Model of Disability' in Helen Spandler, Jill Anderson and Bob Sapey (eds), *Madness, Distress and the Politics of*

2.4 *Beyond the Social Model of Disability*

Despite its wide influence, the social model of disability has never been universally accepted by people with disability, or their supporters and carers, and has been subject to widespread criticism.[36] Debate and confusion around how the social model is meant to operate, how many 'versions' of it there are, and what it really means[37] have fed much of the criticism. However, it has been largely defined by way of opposition to the medical model, with the social model locating disability in society and the medical model in the individual.[38] Commentators sometimes refer to a 'strict'[39] or 'strong'[40] version of the social model, and it is this strict version – which is the one that has proliferated in popular advocacy – that attracts most of the criticism. According to this version, impairment and disability are separate concepts, and disability is caused solely by the existence of ableist prejudices and barriers and their effect on impairment.[41]

2.4.1 Strict Social Model Does Not Reflect Embodied Experience of Impairment

What I will refer to as the strict version of the social model – that disability is *entirely* a product of environment – has been criticised for discounting or ignoring the reality or experience of impairment.[42] It potentially denies the real difference that people

Disablement (Bristol University Press, 2015) 57, 66; D Christopher Ralston and Justin Ho, 'Introduction: Philosophical Reflections on Disability' in D Christopher Ralston and Justin Ho (eds), *Philosophical Reflections on Disability* (Springer, 2010) 1, 4.

36 Barbara Arneil, 'Disability in Political Theory versus International Practice: Redefining Equality and Freedom' in Barbara Arneil and Nancy J Hirschmann (eds), *Disability and Political Theory* (Cambridge University Press, 2016) 10, 26.

37 Mitra suggests that there are at least nine different versions of the social model: Sophie Mitra, 'The Capability Approach to Disability' (2006) 16(4) *Journal of Disability Policy Studies* 236, 237. Commentators describe, for example, a 'minority rights' version of the social model as being prevalent in the United States: Lawson and Priestley (n 33) 3, 5.

38 Jonas-Sébastien Beaudry, 'Beyond (Models of) Disability?' (2016) 41(2) *Journal of Medicine and Philosophy* 210, 210.

39 Ralston and Ho (n 35) 2; referring to the 'strict materialist view': Simo Vehmas and Pekka Mäkelä, 'The Ontology of Disability and Impairment: A Discussion of the Natural and Social Features' in Simo Vehmas, Kristjana Kristiansen and Tom Shakespeare (eds), *Arguing about Disability: Philosophical Perspectives* (Taylor and Francis, 2008) 42, 43; refers to the 'strict constructionist argument'. Bryan S Turner, *Vulnerability and Human Rights* (Pennsylvania State University Press, 2006) 101–2; Andrea Broderick, *The Long and Winding Road to Equality and Inclusion for Persons with Disabilities: The United Nations Convention on the Rights of People with Disabilities* (Intersentia, 2015) 78.

40 Emily Julia Kakoullis and Yoshikazu Ikehara, 'Article 1: Purpose' in Ilias Bantekas et al (eds), *The UN Convention on the Rights of Persons with Disabilities: A Commentary* (Oxford University Press, 2018) 35, 38; Jackie Leach Scully, 'Disability and the Thinking Body' in Simo Vehmas, Kristjana Kristiansen and Tom Shakespeare (eds), *Arguing about Disability: Philosophical Perspectives* (Taylor and Francis, 2008) 57, 58.

41 Clifton (n 6) 10–11.

42 Degener, 'A New Human Rights Model of Disability' (n 29) 41, 47; Lawson and Priestley (n 33) 7–8; Kakoullis and Ikehara (n 40) 38; Beaudry, 'Beyond (Models of) Disability' (n 38) 212;

with disability may experience through their impairment, apart from any real or perceived failure of social or environmental accommodation or change.[43] Critics of the premise that disability is purely caused by environment consider it inadequate at explaining the lived experience of those with disability and realising their rights.[44] For some people, the experience of impairment or its manifestations (e.g. pain, discomfort and increased morbidity) is real and lived, and needs to be recognised as different.[45] As Shakespeare writes, 'in most cases, disabled people are experiencing both the intrinsic limitation of impairment, and the externally imposed social discrimination'.[46] Kong writes that these 'differences that are residual from impairment', even after supports are provided and barriers removed, 'must be taken into account'.[47] In its ostensible and outright rejection of a medical model, the social model has also been criticised for minimising or rejecting the need for prevention of disability and for rehabilitation.[48]

2.4.2 Cognitive Disability and the Challenges of the Social Model

The social model originally developed in the context of advocacy groups that consisted predominantly of people with physical disability,[49] and some commentators have criticised it for not providing a useful model when it comes to responding to cognitive disability. Daniels et al acknowledge that the social model in its strict

Katrine Del Villar, 'Should Supported Decision-Making Replace Substituted Decision-Making? The Convention on the Rights of Persons with Disabilities and Coercive Treatment under Queensland's Mental Health Act 2000' (2015) 4(2) *Laws* 173, 182.

43 Jenny Morris, 'Impairment and Disability: Constructing an Ethics of Care that Promotes Human Rights' (2001) 16(4) *Hypatia* 1, 9–11.

44 Kakoullis and Ikehara (n 40) 36.

45 Jonas-Sébastien Beaudry, 'Theoretical Strategies to Define Disability' in David T Wasserman and Adam Cureton (eds), *The Oxford Handbook of Philosophy and Disability* (Oxford University Press, 2019), 6; the social model 'overlooks aspects of pain, fatigue and sometimes illness relating to certain impairments': Lorella Terzi, 'Vagaries of the Natural Lottery? Human Diversity, Disability, and Justice: A Capability Perspective' in Kimberley Brownlee and Adam Cureton (eds), *Disability and Disadvantage* (Oxford University Press, 2009) 86, 91–92. Margaret Price explores the tension between 'desiring' disability and the reality and problem of pain in Margaret Price, 'The Bodymind Problem and the Possibilities of Pain' (2015) 30(1) *Hypatia* 268, 272.

46 Shakespeare, *Disability Rights and Wrongs* (n 35) 41.

47 Camillia Kong, *Mental Capacity in Relationship: Decision-Making, Dialogue and Autonomy* (Cambridge University Press, 2017) 152; Scully suggests similarly that disability is a set of complex interactions between the 'experience of anomalous embodiment', or 'being physically unusual', and social and cultural factors: Scully (n 40) 60.

48 Arneil (n 36) 30; Shakespeare, *Disability Rights and Wrongs* (n 35) 29; Degener, 'A New Human Rights Model of Disability' (n 29) 52; in the CRPD there is no absolute right to prevention of disabilities but only a right to prevent further disabilities: Article 25 – the Right to Health.

49 Shakespeare, *Disability Rights and Wrongs* (n 35) 2; Anne Plumb, 'UN Convention on the Rights of Persons with Disabilities: Out of the Frying Pan into the Fire? Mental Health Service Users and Survivors Aligning with the Disability Movement' in Helen Spandler et al (eds), *Madness, Distress and the Politics of Disablement* (Bristol University Press, 2015) 183, 184.

form may be useful in framing and therefore addressing (often through technology) the needs of people with a very narrowly defined, but prevalent, range of specific motor and sensory impairments.[50] However, they comment that: 'For significant mental illness and for many brain injuries, neither existing technology nor robust social inclusion can make up for the disability that results'.[51] Plumb (identifying as a survivor of psychiatry) describes how a person experiencing psychosis may be exercising will or agency, but their preferences and actions are based on assumptions that are not just unusual or eccentric but qualitatively different. The concepts of self-determination and autonomy, which Plumb describes as being central to the social model, are problematic for those experiencing these 'non-ordinary' states.[52]

2.4.3 Mixed Models: 'Interactive', 'Biopsychosocial', and 'Multifactorial'

These criticisms have led to arguments for recognising impairment or disability as 'difference', but a difference that should not lead to inequality or social exclusion. Kayess and Sands write, 'that impairment is valued as part of human diversity and human dignity, and therefore people with disability are critical to all aspects of life'.[53] Morris explains why disability activists often deny difference, but also why and how difference should be asserted. She explains that:

> our difference has been the gateway to a denial of human and civil rights: to be recognized as 'severely disabled,' for example, all too often leads to segregation in an institutional setting. We need to change this situation so that the recognition of our difference becomes the gateway to the provision of what we require in order to access our human and civil rights. We have to recognize that disabled people will not get access to full human and civil rights by being treated the same as non-disabled people.[54]

In other words, it is not the recognition of difference in itself that leads to rights-denying behaviour but the nature of the social response that ensues. She affirms, however, that difference needs to be recognised before an appropriate and effective human rights response can be designed and provided.

Many critics of the social model have been advocating for its replacement by a more complex one whereby disability is understood as an 'interaction between medical, psychological, environmental, economic and political factors', some of

[50] Norman Daniels, Susannah Rose and Ellen Daniels Zide, 'Disability, Adaptation and Inclusion' in Kimberley Brownlee and Adam Cureton (eds), *Disability and Disadvantage* (Oxford University Press, 2009) 54, 80.

[51] Ibid 81; see also Peter Bartlett, 'The United Nations Convention on the Rights of Persons with Disabilities and Mental Health Law' (2012) 75(5) *The Modern Law Review* 752, 761.

[52] Plumb (n 49) 186, 187, 194.

[53] Rosemary Kayess and Therese Sands, *Convention on the Rights of Persons with Disabilities: Shining a Light on Social Transformation* (UNSW Social Policy Research Centre. 2020) 46.

[54] Morris (n 43).

which are within the individual and some embedded in the environment.[55] In 2011, the World Health Organization redefined disability as the 'dynamic interaction between health conditions and contextual factors, both personal and environmental', explaining that this reflected a 'biopsychosocial model of disability'.[56] Shakespeare adopts what he refers to as a 'multi-factorial' concept of disability, which recognises the myriad factors that contribute to the 'complexity of disability'.[57]

2.4.4 The 'Human Rights Model' of Disability

The UN Disability Committee in its General Comments describes the CRPD as promoting a 'human rights model' or a 'human-rights-based' model of disability.[58] The human rights model has been described by Degener and endorsed by Kayess (former Chairs of the UN Disability Committee)[59] as an 'alternative to' and 'improvement on the social model of disability' to be used as 'a tool to implement the CRPD'.[60] The human rights model has six features, with two of them prescribing relevantly that 'impairment matters' and that 'identity is composed of different layers'.[61] Under this human rights model, 'impairment' is used

[55] Paul Harpur and Heather Douglas, 'Disability and Domestic Violence: Protecting Survivors' Human Rights' (2014) 23(3) *Griffith Law Review* 405, 415; 'even in the disability movement there are disagreements about this strong version of the social model': Wallcraft and Hopper (n 25) 85; a 'growing number of disability scholars' understand disability as 'the interactoin between the body/mind and the social context. Disability thus exists in the interactive processes of bodies living in social space': Arneil and Hirschmann (n 17) 15.

[56] Arneil (n 36) 26–7; see also reference in: Suzanne Cahill, *Dementia and Human Rights* (Policy Press at the University of Bristol, 2018) 33.

[57] Shakespeare (n 19) 19–21; a more interactive model of disability has been dominant in Scandinavian countries, referred to as the Nordic 'relational' approach, 'which stresses the complex and situational interactions between individual factors and the environment': Traustadóttir (n 29) 12.

[58] *General Comment No 1* [3]; Committee on the Rights of Persons with Disabilities, *General Comment No 4: On the Right to Inclusive Education*, UN Doc CRPD/C/GC/4 (25 November 2016) [4(a)], [36], [44], [62], [71]; Committee on the Rights of Persons with Disabilities, *General Comment No 5: On Living Independently and being Included in the Community*, UN Doc CRPD/C/GC/5 (27 October 2017) [60]; Committee on the Rights of Persons with Disabilities, *General Comment No 6: On Equality and Non-Discrimination*, UN Doc CRPD/C/GC/6 (26 April 2018); Committee on the Rights of Persons with Disabilities, *General Comment No 7: On the Participation of Persons with Disabilities, Including Children with Disabilities, through their Representative Organizations, in the Implementation and Monitoring of the Convention*, UN Doc CRPD/C/GC/7) [1], [60], [76].

[59] As of the time of writing Rosemary Kayess has been appointed Vice Chair until 2026.

[60] Degener, 'A New Human Rights Model of Disability' (n 29) 41; Kayess and Sands (n 53) 2, 9, 20, 21, 29–34, 37.

[61] The six features are: 'First, the human rights model ensures that no person with a disability is denied legal capacity. Second, it goes beyond non-discrimination rights and includes first and second generation human rights. Third, it acknowledges that impairment matters in the life of persons with disabilities. Fourth, it recognizes that identity is composed of different layers. Fifth, it provides a roadmap for non-discriminatory preventative health policy. Sixth, the human rights model proves a roadmap for disability inclusive development and humanitarian aid': Degener, 'A New Human Rights Model of Disability' (n 29) 56.

descriptively, and to the extent that it has normative implications, the model is celebratory of impairment, with Degener describing how the human rights model 'values impairment as part of human diversity and human dignity'.[62]

The human rights model thus responds to the criticisms of the social model by recognising the lived experience of impairment.[63] Of note, though, is that the human rights model includes as its first feature, the recognition of universal legal capacity[64] – an inclusion I argue that fails to recognise the difference of cognitive impairment. For this reason, the human rights model contains internal contradictions and, as a reference point for interpreting and applying Article 12, fails to remedy the limitations of the social model. Although it asserts that 'impairment matters', in the specific case of decision-making ability it fails to acknowledge that decision-making impairment as a matter of fact has an adverse impact on decision-making ability or capacity.

2.4.5 Commentary and Conclusion on Models of Disability and the CRPD

The charity model of disability situated people with disability as objects of pity and discretionary beneficence rather than as subjects of rights, and the dominance of a medical model meant that if a person could not have his or her disability 'fixed' or 'cured', then they were somehow lesser persons. The social model, therefore, developed as an important advocacy tool to accommodate impairment and to include people with disability as subjects of rights. However, a strict version of the social model emphasises that people with disability – once environments have been adapted to support them – are intrinsically 'the same' as each other and the same as those without disability. I argue further in Chapter 6 that this assumption of sameness risks treating people with disability identically, even when their impairment demands differential supports and services for them to achieve equality and social inclusion.[65]

In its effort to de-stigmatise disability, the strict social model risks denying difference, both the different experience of impairment and the disadvantages associated with impairment or disability.[66] I argue that it is important and useful for impairment to be recognised descriptively as atypical or 'non-ordinary' so that it can be responded to in a way that will ensure human rights are fulfilled. There is a plethora of terms (e.g. biopsychosocial, multi-factorial, interactive) to describe disability as something created by ableist social and environmental responses and also by the concrete reality of embodied, lived impairment. There is no consensus on what any new model should be called, so I choose to refer to a social model that recognises the

[62] Kayess and Sands (n 53) 9.
[63] Degener, 'A New Human Rights Model of Disability' (n 29) 56.
[64] Ibid.
[65] See Chapter 6, especially Section 2.
[66] See for example Chapter 3, Section 4.2.1, which describes the socio-economic disadvantages disproportionately associated with disability.

residual impacts of impairment.[67] In using this terminology, I seek to acknowledge both the social determinants of disability and the lived experience of impairment, even after all requisite and available social and environmental supports have been provided. This terminology is also consistent with the description of disability in the CRPD as: 'an evolving concept … that … results from the interaction between persons with impairments and attitudinal and environmental barriers that hinders their full and effective participation in society on an equal basis with others'.[68]

3 DECISION-MAKING AND PEOPLE WITH COGNITIVE DISABILITY

The three dominant models of disability – charity, medical and social – have been used to explain the development of different approaches to decision-making for, with and by adults with cognitive disability. Historically, decisions were made for adults by the institutions where they lived; but, with a move to de-institutionalisation, personal guardians began to be appointed to make decisions for adults in recognition of their individual needs and interests. Guardianship frameworks, in turn, have evolved from strictly paternalistic models to ones that increasingly recognise adults' autonomy. The dominance of guardianship is, in turn, starting to be challenged by the development of supported decision-making practices that endeavour to enable adults with cognitive disability to make their own autonomous decisions.

The next three subsections trace this development in decision-making practices in relation to people with cognitive disabilities – from the routine imposition of institutionalisation to the advent of supported decision-making.

3.1 *The Era of Mass Institutionalisation*

The dominance of the charity and medical models of disability has become associated with the historical practice of routinely 'placing' adults with cognitive disability in institutions run by the State or church, where they resided until death.[69] In these cases, the institution made all decisions for the adults, who had no choice as to where they would live, what they would eat, what (if any) health care would be

[67] See 'differences that are residual from impairment must be taken into account' and 'residual difficulties caused by bodily impairment': Kong, *Mental Capacity in Relationship* (n 47) 152.

[68] *CRPD* Preamble para 5; and this definition is reiterated with minor changes in Article 1; Broderick has written that this definition reflects what she calls a 'social-contextual model of disability', which appears to approximate what I have called a social model that acknowledges the residual impacts of impairment. See: Broderick, *The Long and Winding Road to Equality and Inclusion for Persons with Disabilities* (n 39) 77–9.

[69] Ron McCallum, *The United Nations Convention on the Rights of Persons with Disabilities: An Assessment of Australia's Level of Compliance* (Royal Commission into Violence, Abuse, Neglect and Exploitation of People with Disability, 2020) 91.

provided or the friends or family with whom they would associate. Such adults were truly denied autonomy in their lives in that they were not permitted any personal space over which they could rule nor allowed to make decisions independently, according to their own values. In 2005, an Official Visitor to one such institution in Australia wrote:

> I found an institution used mainly as a place for keeping certain people out of circulation. Remedial treatment and turnover were practically nil. Patients were eking out a 24-hour daily existence in impoverished conditions and some practically in squalor. Many requests for discharge were ignored and not worth recording. Paramedics were unknown.[70]

In the 1980s and 1990s, Australia and other Western nations began de-institutionalising people with disability by moving them into community living.[71] This move was driven by revelations of significant abuse and exploitation behind closed walls,[72] and a belief that de-institutionalisation would increase the adults' autonomy and lead to fewer human rights abuses.[73] I note that while the era of mass institutionalisation has ostensibly ended, in reality institutionalisation continues, with significant concerns prevailing about the lack of appropriate community-living options for people with disability.[74]

[70] Office of the Public Advocate (Qld), Submission No 73 to Senate Standing Committee on Community Affairs, Parliament of Australia, *Inquiry into Violence, Abuse and Neglect against People with Disability in Institutional and Residential Settings, Including the Gender and Age Related Dimensions, and the Particular Situation of Aboriginal and Torres Strait Islander People with Disability, and Culturally and Linguistically Diverse People with Disability* (June 2015) 5 ('*Submission to Senate Standing Committee*'); see also description of people being 'mopped down' in an institution in the 1960s: Raimond Gaita, *A Common Humanity: Thinking about Love & Truth & Justice* (Text Pub, 2nd ed, 2000) 17–18.

[71] Frances Owen and Gillian MacKinnon, 'The Right to Community Living' in Dorothy Griffiths, Frances Owen and Shelley L Watson (eds), *The Human Rights Agenda for Persons with Intellectual Disabilities* (NADD, 2012) 39.

[72] Office of the Public Advocate (Qld), *Submission to Senate Standing Committee* (n 70).

[73] Mary Donnelly, 'Deciding in Dementia: The Possibilities and Limits of Supported Decision-Making' (2019) 66 *International Journal of Law and Psychiatry* 101466: 1–7; given the alternative motivations of cost-reduction, de-institutionalisation in itself has not always resulted in improved living conditions or greater choices of support services. This is despite the fact that one of the aims of de-institutionalisation was to increase the autonomy of adults with disabilities: see Jonathan Herring, *Caring and the Law* (Hart Publishing, 2013) 91, 146; Allen Buchanan and Dan W Brock, *Deciding for Others: The Ethics of Surrogate Decision Making* (Cambridge University Press, 1989) 313–15. Also relevant was the development of psychotropic drugs, which could be used to treat (but some would still say 'contain') adults within community settings: Owen and MacKinnon (n 71) 41–2. For use of psychotropic drugs as a form of containment, see Office of the Public Advocate (Qld), *Upholding the Right to Life and Health: A Review of the Deaths in Care of People with Disability in Queensland* A Systemic Advocacy Report (February 2016) 47.

[74] McCallum (n 69) 91–9; Disability Royal Commission, *Group Homes* (Issues Paper, 28 November 2019); Disability Royal Commission, *Overview of Responses to Group Homes* (Issues Paper, September 2020).

3.2 *Legal Capacity Tests*

Under current guardianship regimes, a decision can generally be made for an adult when the adult lacks decision-making capacity or legal capacity. As the ALRC explains, 'Legal capacity sets the threshold for individuals to take certain actions that have legal consequences' and 'Legal capacity goes to the validity, in law, of choices and being accountable for the choices made'.[75] So, if an adult is not accorded legal capacity for a decision, any decision the adult makes may not be legally enforceable. Capacity has been described as the 'gateway to autonomy'[76] so that if a person is found to lack legal capacity, another person (i.e. a substitute – whether a guardian, surrogate, representative, or clinician in health-service provision) may be empowered to make a decision for them that will be legally recognised and for which the substitute will be legally accountable. If a substitute is making a decision for an adult in this way, the substitute is ostensibly or potentially stepping into the adult's own sovereign, personal space, and compromising or denying the adult's autonomy.[77] The three commonly described tests for capacity are the status, outcome and functional tests, with the functional test generally seen as showing greater respect for autonomy than the other two.[78]

3.2.1 Status Test

Legal capacity in some cases is said to be determined by status – for example, a person currently lacks legal capacity if they are under eighteen years of age; historically, both women[79] and slaves lacked capacity by virtue of their status as such.[80] An example of a capacity test that is *partially* status based can be found in the Mental Capacity Act 2005 (England and Wales) ('MCA'). Under the first limb of this test, a guardian can only be appointed for a person who is unable to make a decision 'because of *an impairment of or disturbance* in the functioning of, the *mind or brain*' [emphasis added];[81] with the MCA's Code of Practice providing diagnostic examples of '…a disability, condition or trauma …'

[75] ALRC (n 33) 44.

[76] Mary Donnelly, *Healthcare Decision-Making and the Law: Autonomy, Capacity and the Limits of Liberalism* (Cambridge University Press, 2010) 2.

[77] See Chapter 4.

[78] Lucy Series and Anna Nilsson, 'Article 12 CRPD: Equal Recognition before the Law' in Ilias Bantekas, Michael Ashley Stein and Anastasiou Dimitris (eds), *The UN Convention on the Rights of Persons with Disabilities: A Commentary* (Oxford University Press, 2018) 339, 356; Suzanne Doyle and Eilionóir Flynn, 'Ireland's Ratification of the UN Convention on the Rights of Persons with Disabilities: Challenges and Opportunities' (2013) 41(3) *British Journal of Learning Disabilities* 171, 174.

[79] Law Commission, *Mental Incapacity* (Report No 231, 1995) 32.

[80] Anna Arstein-Kerslake et al, 'Future Directions in Supported Decision-Making' (2017) 37(1) *Disability Studies Quarterly* 5–5.

[81] *Mental Capacity Act 2005* (England and Wales) ('*MCA*') s 2(1); see also, 'a person in need of a guardian' is defined as, 'a person who because of a disability, is totally or partially incapable of managing his or her person', *Guardianship Act 1987* (NSW) ('*GA NSW*') s 3; in Tasmania a guardian or administrator can be appointed 'for a person with disability', *Guardianship and*

The status-based approach to determining capacity aligns most clearly with a medical model of disability, and in some jurisdictions mere determination of medical conditions or identification of impairments can be enough to rebut the presumption of capacity and confer a status of mental and legal incapacity.[82] The status approach lay behind what was historically the default model of 'plenary' guardianship, which had and still has the effect of depriving the adult of decision-making power in all areas of life.[83] Moreover, an overturning of that determination would be extremely unlikely, particularly as the individual might be denied legal standing to challenge it. Under this model, an individual could therefore be said to have the 'status' of disability and incapacity with their autonomy being significantly undermined or denied. A status test for capacity on the basis of impairment or disability is also widely perceived to be discriminatory and in breach of Article 12.[84]

3.2.2 Outcome Test

An outcome-based approach to determining capacity considers the probable outcome of a particular decision if it were to be made by the adult, and effectively works backwards to determine if the adult has capacity to make the decision.[85] Examples of an outcome-based test can be found in legislation that allows for the appointment of a guardian if an adult is unable to make 'reasonable' decisions or judgements.[86]

Administration Act 1995 (Tas) ('*GAA Tas*') s 20 (1); in South Australia the definition of 'mental incapacity' includes a test of inability 'as a result of … damage to, or any illness, disorder, imperfect or delayed development, impairment or deterioration of the brain or mind'. *Guardianship and Administration Act 1993* (SA) ('*GAA SA*') s 3(1); the legislation in the Australian Capital Territory sets a condition that 'the person's decision-making ability is impaired because of a physical, mental, psychological or intellectual condition or state, whether or not the condition or state is a diagnosable illness'. *Guardianship and Management of Property Act 1991* (ACT) ('*GMPA ACT*') s 5. By way of contrast, the *Guardianship of Adults Act 2016* (NT) ('*GAA NT*') s 5(5) expressly rejects a status-based test when it provides that for the purposes of guardianship appointment: 'The cause of the impairment of an adult's decision-making capacity is immaterial'.

[82] Ben White, Lindy Willmott and Shih-Ning Then, 'Chapter 7: Adults Who Lack Capacity: Substitute Decision Making' in Ben White, Fiona McDonald and Lindy Willmott (eds), *Health Law in Australia* (Lawbook Co, 3rd ed, 2018) 207, 230.

[83] Terry Carney, 'Searching for Workable Alternatives to Guardianship for Vulnerable Populations?' (2015) 1(1) *Ethics, Medicine and Public Health* 113, 115; John Chesterman, 'The Review of Victoria's Guardianship Legislation: State Policy Development in an Age of Human Rights' (2010) 69(1) *Australian Journal of Public Administration* 61, 61. Further, a determination of incapacity could mean that an adult was stripped of civil participation in all of its facets so as to be unable to vote or run for public office: Quinn and Arstein-Kerslake (n 19) 44.

[84] *General Comment No 1* [15]; Martin et al, *Achieving CRPD Compliance* (n 4) 31.

[85] White, Willmott and Then (n 82) 231.

[86] Ibid; e.g., the Western Australian legislation has as a precondition for a guardian that a person is 'unable to make reasonable judgements in respect of any treatment proposed to be provided'. *Guardianship and Administration Act 1990* ('GAA WA') ss 110ZD(1), 110ZG(1), 110ZJ(1); and simlarly the second limb of the test in the Tasmanian legislation is that a person 'is unable … to make reasonable judgements in respect of all or any matters relaing to his or her person or circumstances': *GAA Tas* s 20(1)(b).

The test of reasonableness tends to focus on the effect of the decision itself rather than on the person's decision-making ability in proceduralist terms.[87] When it comes to health decisions, this means that greater capacity may be required to refuse treatment than to consent to the same treatment, because refusal is seen to be riskier.

The outcome test has been criticised for holding an adult with cognitive disability to a higher standard of decision-making than those in the wider community, who are indeed free to make bad or unwise decisions.[88] For this reason, it has been widely decried as denying adults' autonomy and what is referred to as their 'dignity of risk' (i.e. the right to made 'bad' or risky decisions).[89] The outcome test infringes a proceduralist account of autonomy because it imposes on the adult an idea of the good that may be outside their own value system.[90] This test has been criticised as the one most profoundly based on lack of respect for the autonomy of the adult, because it pays no heed to their will or preferences.[91] Despite the ongoing existence of outcome tests in some legislation, Quinn wrote in 2010 that 'Nobody seriously suggests an outcomes approach now'.[92]

[87] Clíona de Bhailís and Eilionóir Flynn, 'Recognising Legal Capacity: Commentary and Analysis of Article 12 CRPD' (2017) 13(1) *International Journal of Law in Context* 6, 10; Leslie Salzman, 'Guardianship for Persons with Mental Illness – A Legal and Appropriate Alternative' (2011) 4 *St Louis University Journal of Health Law and Policy* 279, 303. It has applied in some cases to find an adult incapacitous when they've entered a mental health facility voluntarily, only to find that they are involuntarily detained when they wish to exit: Buchanan and Brock (n 73) 217; Donnelly, *Healthcare Decision-Making and the Law* (n 76) 105.

[88] de Bhailís and Flynn (n 87) 10.

[89] For further discussion of the 'dignity of risk', see Chapter 5, Sections 3.2.1 and 4.1; 'It is to recognise the "dignity of risk" so that individuals with impairments are allowed to make unwise choices without paternalistic incursions into their lives': Camillia Kong, 'The Convention on the Rights of Persons with Disabilities and Article 12: Prospective Feminist Lessons against the "Will and Preferences" Paradigm' (2015) 4(4) *Laws* 709, 725; 'The General Comment No 1 states that there must be safeguards to protect against undue influence, yet these must respect the rights, will and preferences of the person, including the right to take risks and make mistakes'; Series and Nillson (n 78) 371.

[90] 'A results-based approach is objectionable because of the inherent contradiction between allowing the majority to make bad mistakes without intervention and disallowing a minority to make the same mistakes and overplaying the protective role of the law …' Gerard Quinn, 'Concept Paper "Personhood and Legal Capacity": Perspectives on the Paradigm Shift of Article 12 of the CRPD' (Conference Paper, HPOD Conference, 20 February, 2010) 14 www.inclusiveirelandie/document/HarvardLegalcapacitygqdraft2.doc; on the dignity of risk, see also Quinn and Arstein-Kerslake (n 19) 41; reflecting the concern that an outcome test denies the dignity of risk, some legislation acknowledges that for an adult 'the right to make decisions includes the right to make decisions with which others may not agree'. See, for example, *Guardianship and Administration Act 2000* (Qld) ('GAA *Qld*') s 5(a); similarly the Victorian legislation provides that, 'it should not be assumed that a person does not have decision-making capacity in relation to a matter merely because the person makes a decision that, in the opinion of others, is unwise': *Guardianship and Administration Act 2019* (Vic) ('GAA *Vic*'); see also *MCA* s 1(4).

[91] Arstein-Kerslake, *Restoring Voice to People with Cognitive Disabilities* (n 29) 85.

[92] Quinn, 'Concept paper "Personhood and Legal Capacity"' (n 90) 13.

3.2.3 Functional Test

The third approach to testing capacity is the functional test, which ostensibly considers only the process by which a decision is made. The functional test differs from a status-based approach in that a person may have capacity to make decisions in some domains – for example, everyday shopping – but may not have the capacity to enter into a complex financial transaction.[93] Under this test, capacity is domain-specific but also time-specific.[94]

The functional test can be found in the MCA as well as in the guardianship legislation of Australian states. A typical version of a functional test can be found in the *Guardianship and Administration Act 2019* (Vic), which requires that the adult be able:

(a) to understand the information relevant to the decision and the effect of the decision; and
(b) to retain that information to the extent necessary to make the decision; and
(c) to use or weigh that information as part of the process of making the decision; and
(d) to communicate the decision …[95]

A functional test of capacity, which assesses whether the adult has the ability to understand the decision to be made, weigh the relevant information and communicate their decision, is consistent with a liberal view of autonomy whereby the adult's own perception of 'the good' must be respected.[96] The functional test is ostensibly a procedural, 'value neutral' test of capacity that doesn't require the adult to hold any particular view or values to be found capacitous.[97] The functional test is also, however, subject to criticism for largely following a medical model, with findings of incapacity being almost entirely determined on the basis of psychiatric evidence.[98] It has also been criticised by the UN Disability Committee and others for being

[93] White, Willmott and Then (n 82) 229–30.

[94] In cases of both mental illness and aged dementia, this temporal limitation if applied carefully can have a significant impact, as capacity can fluctuate on a daily basis.

[95] Section 5; see also definition of 'person in need of a guardian' in *GA NSW* s 3; *GAA Tas* s 6(2); in Queensland, the test for capacity is similar to that above, but includes a third limb, that the person also has to be 'capable of … (b) freely and voluntarily making decisions about the matter', *GAA Qld* sch 4 Dictionary – definition of 'capacity'; *MCA* s 2 retains a status approach as a threshold, with an overlay of a functional test.

[96] Rawls (n 10) 11.

[97] Lucy Series, 'Relationships, Autonomy and Legal Capacity: Mental Capacity and Support Paradigms' (2015) 40 *International Journal of Law and Psychiatry* 80, 81.

[98] Quinn and Arstein-Kerslake (n 19) 45; Sam Boyle, 'Medical Evidence of Capacity in a Legal Setting: To What Extent Do Courts and Tribunals Make Their Own Decisions?' (2018) 25(2) *Journal of Law and Medicine* 572, 580–5. Also, the time-specific aspect of the functional test can be difficult to apply in practice, with many health-care decisions related to ongoing treatment rather than one-off procedures: Donnelly, *Healthcare Decision-Making and the Law* (n 76) 120.

discriminatory, because it impacts disproportionately on adults with cognitive disability (as discussed further in Chapter 6).[99] Behind this conclusion is an assumption that a finding of incapacity and the appointment of a substitute will always be deleterious for the adult and constitute a breach of human rights and Article 12 in particular.[100]

3.3 *Decision-Making Principles*

Once a determination of incapacity is made, legal frameworks may allow for a substitute to make a decision for the adult. This section provides a description of principles that may apply in cases of decision-making by substitutes, from 'best interests' to substituted judgement' and 'will and preferences'.

3.3.1 'Best Interests' Principle

Liberal theory demands that autonomy and self-determination be respected for those with capacity, but the 'best interests' test is used to ensure that adults with impaired capacity do not make decisions that place them or others at high risk of harm.[101] In practice, a decision made by a substitute according to a pure 'best interests' test[102] may deny the adult any role or voice in decision-making. In addition, a substitute could make a decision that he or she believed to be for the well-being of the adult, whether or not the adult objected to it.[103] The best interests test has been widely criticised for its encapsulation of a paternalistic attitude towards adults with cognitive

[99] *General Comment No 1* [15]. See further critique in Renato Antonio Constantino Caycho, 'The Flag of Imagination: Peru's New Reform on Legal Capacity for Persons with Intellectual and Psychosocial Disabilities and the Need for Understandings in Private Law' (2020) 14(June) *The Age of Human Rights Journal* 155, 166; Mary Keys, 'Article 12' in Valentina Della Fina, Rachele Cera and Giuseppe Palmisano (eds), *The United Nations Convention on the Rights of Persons wtih Disabilities: A Commentary* (Springer International Publishing, 2017) 263, 266. For further discussion of the functional test in the context of Article 12 see Chapter 6, Section 4.

[100] The status, outcome and functional tests are described above as if they are separate and distinct, but they are sometimes mixed together and incorporated into multi-level legislative tests, as in the MCA. Commentators have also pointed out that no matter what the legislative requirements may be, in practice the tests overlap, and courts tend to rely heavily on psychiatric evidence when purportedly applying any of the three different tests: Quinn and Arstein-Kerslake (n 19) 45; Donnelly, *Healthcare Decision-Making and the Law* (n 76) 115; also Boyle's view is that under the MCA while outcome cannot be determinative of capacity, it can be considered as *one* factor amongst others: Sam Boyle, 'Is the Wisdom of a Person's Decision Relevant to Their Capacity to Make that Decision?' (2020) 46(1) *Monash University Law Review* 39, 41–7; see also Series (n 97) 82.

[101] Buchanan and Brock (n 73) 21–2.

[102] Section 4 *MCA* provides that in determining a person's best interests, their 'past and present wishes and feelings …' must be considered 'so far as is reasonably ascertainable' – i.e. it softens a traditional, 'pure' best interests test by explicitly requiring consideration of the adult's will and preferences.

[103] ALRC (n 33) 49–50; commentators have also suggested that achievement of self-determination and autonomy in itself may promote well-being: Buchanan and Brock (n 73) 37.

impairment, because it may diminish the freedom of the adult and threaten his or her self-determination.[104] Under the best interests test, an adult whose decisions are made by a substitute may be denied the dignity of risk (i.e. the right to make unwise decisions).[105] The best interests test is still prevalent in legislation in common law jurisdictions,[106] although considered by many to be anachronistic and contrary to a twenty-first-century human rights approach to disability. In particular, the UN Disability Committee is of the firm view that it is contrary to Article 12 CRPD.[107] In imposing external values onto the adult, best interests decision-making is perceived by many as undermining the adult's autonomy and to be discriminatory on the basis of disability.[108]

3.3.2 Substituted Judgement and 'Will and Preferences'

Given the concerns described above, the best interests test has been replaced in some cases with 'substituted judgement'. According to this practice, the substitute must consider what decision the adult would have made had he or she *not* lost capacity and then give effect to their views.[109] In refraining from imposing the substitute's own views on the adult, substituted judgement is less paternalistic than 'best interests' decision-making and places greater weight on the adult's right to self-determination.[110] Most guardianship legislation in Australian states, as well as the MCA, now acknowledge and apply the substituted judgement test, at least to a limited extent.[111]

However, there are many occasions where a person may never have had capacity, most notably in cases of intellectual disability from birth, or in cases of brain injury

[104] ALRC (n 33) 49–50.

[105] For further discussion of the concept of 'dignity of risk' see Chapter 5, Section 4.1.

[106] For example: *GA NSW* s 4(a) states that 'the welfare and interests of … persons should be given paramount consideration', s 16(2A) provides that the tribunal must consider 'best interests' when reviewing an appointment, and s 21A(2) provides that whether a guardian has acted in 'best interests' is one criterion to be considered to grant the guardian immunity, and see also ss 25G, 25P(2)(b) and s 25D in relation to financial management orders; *GAA NT* s 4(1); *GAA WA* s 51; in relation to the MCA see Donnelly, *Healthcare Decision-Making and the Law* (n 76) 179–80; for the United States see: Julia Duffy, 'What if Britney Spears Lived in Australia? Disrupting the Binary Framing of Guardianship versus Supported Decision-Making', (forthcoming 2023) 30 *Transnational Law and Contemporary Problems.*

[107] *General Comment No 1* [21].

[108] Anna Arstein-Kerslake, 'An Empowering Dependency: Exploring Support for the Exercise of Legal Capacity' (2016) 18 *Scandinavian Journal of Disability Research* 77, 84; Arstein-Kerslake and Flynn (n 3) 479.

[109] White, Willmott and Then (n 82) 210–11.

[110] 'the value of self-determination can still be indirectly respected by guidance principles for surrogates that require them to seek to make decisions that are most in accordance with the person's values while competent …': Buchanan and Brock (n 73) 38.

[111] See: 'the principle of substituted judgement' in *GAA Qld* s 11B(3); *GA NSW* ss 4(d), 14(2)(a), 28(2)(a), 40(3)(a), 44(2)(a), 45(5)(a); *GMPA ACT* s 4(2); *GAA Vic* s 8(c); *GAA Tas* s 6(c); *GAA NT* s 4(3)(1); *GAA SA* s 5(a); *GAA WA* s 51.

acquired before the age of eighteen years.[112] Moreover, in cases of acquired brain injury, mental illness or age-related dementia, an adult's values post-injury or illness may change.[113] In increasing recognition of the importance of autonomy for adults with disability, legislation has begun to require that substitutes make decisions that accord with the 'will and preferences' of the adult as expressed at the time that the decision is made.[114] The UN Disability Committee, in its prescription of universal legal capacity, favours the view that Article 12 demands that the adult's will and preferences, as expressed contemporaneously with the decision, be recognised and upheld.[115] This practice recognises the adult as always autonomous, the UN Disability Committee prescribing such autonomy as a pre-condition for recognising personhood.

3.4 *Supported Decision-Making*

In the 1990s, there was a move towards supported decision-making as an adjunct to existing guardianship regimes.[116] Supported decision-making is aimed at maximising an individual's autonomy so that, ideally, an adult who would otherwise have been assessed as not having legal capacity can make decisions that are recognised as their own, and for which they are therefore legally accountable. This movement towards supported decision-making led to the adoption of Article 12(3) CRPD, which provides in full that: 'States Parties shall take appropriate measures to provide access by persons with disabilities *the support they may require* in exercising their legal capacity' [emphasis added].

[112] White, Willmott and Then (n 82) 211. Donnelly opines that in cases where the substituted judgement approach was used by courts for an adult who never had capacity, it was beginning to: 'look more and more like a legal fiction whereby judges made decisions which they considered appropriate under the guise that this was what the patient would have wanted': Donnelly, *Healthcare Decision-Making and the Law* 186.

[113] See Buchanan and Brock (n 73) 155; 'the issue of changing identity following lack of capacity arises' in terms of the moral authority of advance directives, in Donnelly, *Healthcare Decision-Making and the Law* (n 76) 199; note that, 'The profile of people who are the subject of guardianship applications in NSW has … changed. Initially, the largest group was people with intellectual disability. Cases involving people with dementia are now the most common. Cases involving people with a mental illness or brain injury also make up a significant number of applications': New South Wales Law Reform Commission, *Review of the Guardianship Act 1987* (Report No 145, May 2018) xxii.

[114] For example, in Queensland the 'structured decision-making' process appears to provide that it is the will and preferences at the time that the substitute decision is being made that are primarily relevant. It is only when these contemporaneous wishes cannot be determined that the adult's previously expressed will and preferences should be considered: GAA *Qld* s 11B General Principle 10 'Structured decision-making' (1)(4). This potential for tension and conflict is an issue of some significance given that guardianship appointments in Australia and elsewhere are increasingly and disproportionately being made for persons over the age of 65 years.

[115] The exception to this is that the UN Disability Committee *does* endorse the use of advance planning instruments, or advance directives: *General Comment No 1* [17].

[116] Then (n 25) 134.

Under a strict social model, an impairment is only framed as a disability because our ableist environment fails to accommodate it.[117] So, in the case of decision-making, the provision of social and environmental supports minimises the impact of cognitive impairment on decision-making ability so that legal capacity may be achieved and recognised. Adults who would otherwise be considered to lack capacity or decision-making ability can be supported by assistants, mentors, translators, technology or education to make legally effective and recognised decisions. In this way, they can be recognised as having legal capacity on an equal basis with others. While there is contention as to whether or not supported decision-making can or should totally replace decision-making by substitutes, there is nevertheless broad common ground that Article 12(3) CRPD should apply when feasible.

Support can include enlisting a trusted support person for the adult, advocacy, or communication assistance.[118] The UN Disability Committee has indicated no one-size-fits-all model of supported decision-making but that the diversity of individuals and situations should be respected and taken into account.[119] It provides that a support person 'formally chosen' by an adult must have legal recognition; there must be legal mechanisms to 'challenge the action of a support person', and an adult has a right to refuse support.[120] While practices of supported decision-making are still evolving, provided below is a brief overview of the principles espoused, some practices developed to date, and the ongoing challenges.

3.4.1 Supported Decision-Making in Legislation

Canadian provinces were at the forefront in developing legislative models for supported decision-making even before adoption of the CRPD. For example, the

[117] See discussion of the social model at Section 2.3 of this chapter.

[118] In 2012, Dinerstein referred to Inclusion Europe's position paper on supported decision-making, which prescribes eight key elements of a supported decision-making model: promotion of self-advocacy; 'using mainstream mechanisms …' to protect an adult's best interests; replacing substitute decision-making; formal support and registered supporters only for significant decisions; selection and training of supporters; overcoming communication barriers; preventing conflicts between the adult and their supporter; and the inclusion of safeguards: Dinerstein (n 33) 10, citing Inclusion Europe, *Key Elements of a System of Supported Decision-Making* (Position Paper, 2018) http://inclusion-Europe.org/images/stories/documents/PositionPapers/Position_Supported_Decision_Making_EN.pdf.; Gooding suggests broad principles for supported decision-making as: protects autonomy by offering support appropriate to an individual when he or she is exercising choice; embraces an 'inter-dependent' concept of autonomy, i.e. interdependence between the adult making the decision and a support person; acknowledges the dignity of risk but protects the individual from abuse; respects equality and non-discrimination; and reflects the norms of international human rights law: Piers Gooding, 'Supported Decision-Making: A Rights-Based Disability Concept and Its Implications for Mental Health Law' (2013) 20(3) *Psychiatry, Psychology and Law* 431, 445; see also Chris Bigby, et al, *Diversity, Dignity, Equity and Best practice: A Framework for Supported Decision-making. Royal Commission into Violence, Abuse, Neglect and Exploitation of People with Disability* (2023) https://disability.royalcommission.gov.au/policy-and-research/research-program

[119] *General Comment No 1.*

[120] Ibid [29].

Representation Agreement Act 1996 of British Columbia allows for an adult who may otherwise lack capacity but who has enough 'capability' to appoint a supporter, to do so. The goal is that once the supporter is in place, this person can assist in raising the adult's capability or capacity to a level where the adult can make a range of other decisions.[121] Another early adopter was Sweden where legislation introduced the role of the 'god man' or mentor. The 'god man' is a support person and decision-making assistant appointed informally or by a tribunal with the consent of the adult with cognitive disability.[122] Since ratification of the CRPD, legislation formally recognising supported decision-making has been enacted in several other countries including the Republic of Ireland,[123] the United States,[124]

[121] *Representation Agreement Act 1996* (BC) s 8. Although enacted almost 30 years ago, the Act provides a useful example of typical legislative scaffolding incorporated in the increasing number of supported decision-making statutes. The agreement with the supporter is called a representation agreement, which is a novel combination of a supported decision-making agreement and enduring power of attorney. The effect of the legislation is that supporters or representatives have the same rights to access information relating to the adult as the adult themselves (s 10); protective factors include exclusion of real property deals (s 2); representatives/supporters have obligations to keep accounts (s 16(8)); and there are requirements for an additional representative or 'monitor' in the case of financial matters, and oversight by the Public Trustee (s 12). Other examples of Canadian legislation are the *Adult Guardianship and Trusteeship Act 2008* (Alberta), which allows for both supported decision-making and co-decision making (ss 4, 13); and the *Adult Guardianship and Co-Decision-Making Act* 2000 (Saskatchewan), which allows for appointment of a co-decision-maker if an adult's capacity is impaired so that he or she requires assistance to make 'reasonable' decisions (s 14(a)(i)). For further description and analysis of this and other Canadian legislation, see Julia Duffy and Kelly Unsworth, 'Supported Decision-Making, Legal Risk and Commercial Uncertainty' (Conference Paper, Australian Guardianship and Administration Council National Conference: Reflecting Will and Preference in Decision-Making, 17–18 October 2016) <https://agac.org.au/images/stories/2016nsw/duffy-et-al-supp-dec-mk-legrsi-comm-unc.pdf>; also, Then (n 25) 151–4.

[122] Usually, the appointment is of a friend or relative, but professionals can also be appointed and paid a small fee. The introduction of this concept of a legally recognised mentor avoids the need for a determination of incapacity and has served to greatly decrease the appointment of 'forvaltare', i.e. guardians or administrators: Office of the Public Advocate (Qld), *Decision-Making Support and Queensland's Guardianship System: A Systemic Advocacy Report* (A Systemic Advocacy Report, April 2016) 167; Arstein-Kerslake and Flynn (n 3) 478; Christine Bigby et al, 'Delivering Decision Making Support to People with Cognitive Disability – What Has Been Learned from Pilot Programs in Australia from 2010 to 2015' (2017) 52(3) *Australian Journal of Social Issues* 222, 223; Gooding (n 118) 444; Jacinta Douglas et al, 'Factors That Underpin the Delivery of Effective Decision-Making Support for People with Cognitive Disability' (2015) 2(1) *Research and Practice in Intellectual and Developmental Disabilities* 37, 39.

[123] *Assisted Decision-Making (Capacity) Act 2015* (Republic of Ireland) s 10 provides that an adult who considers that their capacity 'is in question or may shortly be in question' can appoint a person to assist them in making decisions on personal welfare or property matters.

[124] In Texas, legislation was introduced in 2015 that provides for the recognition of supported decision-making agreements and includes some immunity for third parties acting in good faith who rely on the agreement: Eliana J Theodorou, 'Supported Decision-Making in the Lone-Star State' (2018) 93(4) *New York University Law Review* 973, 996; see also California: 'What if Britney Spears Lived in Australia? Disrupting the Binary Framing of Guardianship versus Supported Decision-Making' (forthcoming 2023) 30 *Transnational Law and Contemporary Problems*; as of

and Peru,[125] and there have been some limited endeavours to introduce legislation for supported decision-making in the Australian State of Victoria.[126] However, it is noteworthy that none of these legislative models has functioned so as to abolish decision-making by substitutes in all cases.[127] Some further analysis of the limitations of legislative regimes is revisited in later chapters.[128]

3.4.2 Informal Support Trials in Australia

Australia has been described as a leader in its development of supported decision-making trials outside of legislative frameworks, with trials led by government and academics in several Australian states and territories.[129] Results from supported decision-making trials have shown overall that the participants completing the trials had 'increased confidence, interpersonal skills, greater self-determination, success within the workplace, the ability to sustain important relationships' and sometimes the ability to transition from a guardianship order.[130] All of this would point to increased autonomy for the participants, consistent with the policy aims. However, the trials have also revealed challenges, which include conflicts between supporters and others involved in the adult's life[131] and reluctance by service providers to

October 2022 15 states and the District of Columbia had passed supported decision-making laws: sdmy Supported Decision-Making New York, *SDMA Legislation in the US and Elsewhere* web page, 11 October 2022) https://sdmny.org/supported-decision-making-legislation/supported-decision-making-agreement-legislation-in-the-u-s-and-elsewhere/

[125] In 2018, Peru repealed antiquated guardianship laws and replaced them with legislation that recognises a decision-making supporter appointed by an adult, and also allows for court appointment of supporters where an adult cannot communicate their will and preferences: Antonio Martiniez-Pujalte, 'Legal Capacity and Supported Decision-Making: Lessons from Some Recent Legal Reforms' (2019) 8(3) *Laws* 1.

[126] Allows for 'supportive guardianship orders': *GAA Vic*; allows for a person to appoint 'another person to support the person and represent the interests of the person in making medical treatment decisions': *Medical Treatment Planning and Decisions Act 2016* (Vic); allows for the appointment of a 'supportive attorney': *Powers of Attorney Act 2014* (Vic).

[127] No law reform agency in English-speaking jurisdictions has recommended complete abolition of decision-making by substitutes: Shih-Ning Then et al, 'Supporting Decision-Making of Adults with Cognitive Disabilities: The Role of Law Reform Agencies – Recommendations, Rationales and Influence' (2018) 61 *International Journal of Law and Psychiatry* 64, 69; also, the legislation in Peru has been criticised for failing to completely abolish decision-making by substitutes: Caycho (n 99) 162–4.

[128] See Chapter 4, Section 4.5 and Chapter 6, Section 5.1.

[129] Kristin Booth Glen, 'Supported Decision-Making from Theory to Practice: Further Reflections on an Intentional Pilot Project' (2020) 13 *Albany Government Law Review* 94, 104; Bigby et al (n 122) 224; Terry Carney et al, 'Realising "Will, Preferences and Rights": Reconciling Differences on Best Practice Support for Decision-Making?' (2019) *Griffith Law Review* 357, 365–75; Office of the Public Advocate (Qld), *Decision-Making Support and Queensland's Guardianship System* (n 122).

[130] Office of the Public Advocate (Qld), *Decision-Making Support and Queensland's Guardianship System* (n 122) 172; Bigby et al (n 122) 231–2.

[131] Bigby et al (n 122) 233; a supported decision-making trial in New York City included mediation services as a component: Glen, 'Supported Decision-Making from Theory to Practice' (n 129) 97, 115–16.

recognise decisions or acknowledge supporters.[132] One trial revealed that some degree of interpretation was always required to determine an adult's will and preferences, suggesting 'a fine line between realisation of will and preference and substitution for it'.[133] Supported decision-making trials have yet to be attempted with adults with severe cognitive disability, leaving it unknown or unclear how those hard cases are to be dealt with in practical terms.[134]

3.4.3 Advance Planning

Advance planning directives are typically understood to be written directives made by an adult when they have capacity. The directives either appoint a person to make decisions for the adult at a future time when they have impaired capacity or provide specific directions for how their health or other needs should be met at that future time.[135] *General Comment No 1* endorses the use of advance planning directives as a tool for supported decision-making. There is broad agreement that they can further the autonomy and self-determination of adults with cognitive disability as required by Article 12.[136]

3.5 *Summary and Conclusions*

This section has mapped out a brief history of legal and policy frameworks for decision-making for, with and by adults with cognitive disability from the era of mass institutionalisation to current practices of decision-making by substitutes and supported decision-making. Although policy frameworks have shifted from promoting paternalism to valuing autonomy, it is important to note that implementation is still a work in progress and that neglect and abuse of many adults with cognitive disability is still a grim reality.

Nevertheless, the trajectory of development from institutionalisation to supported decision-making reflects the increasing endeavours of policymakers to recognise

[132] Office of the Public Advocate (Qld), *Decision-Making Support and Queensland's Guardianship System* (n 122) 173.

[133] Carney et al (n 129) 169; see also description below of use of the 'best interpretation' principle in supported decision-making at Section 4.1 and criticisms of it at Section 4.2.5.

[134] Bigby et al (n 122) 235.

[135] White, Willmott and Then (n 82) 217–27; Lindy Willmott, Ben White and Ben Matthews, 'Law, Autonomy and Advance Directives' (2010) 18 *Journal of Law and Medicine* 366.

[136] *General Comment No 1* [17]; Jill Stavert, 'Supported Decision-Making and Paradigm Shifts: Word Play or Real Change?' (2020) 11 *Frontiers in Psychiatry* 571005: 1–9; however, *General Comment No 1* only endorses advance directives if they are *not* triggered by a determination of incapacity but by the occurrence of some other event or condition. The UN Disability Committee's view is that a trigger not associated with disability or incapacity (i.e. a disability 'neutral' trigger) is allowable under Article 12. In such a case, an advance directive could deny legal capacity, but not discriminatorily on the basis of disability. However, I argue in Chapter 6 (Sections 4.1 and 4.2) that relying on a concept of disability 'neutrality' fails to interrogate what equality means when we are all situated differently and when we recognise the residual impacts of impairment.

the personhood and autonomy of adults with cognitive disability. This, in turn, reflects a desire to respond to adults' needs in ways that respect their rights to non-discrimination and equality. This growing focus on autonomy and equality has been explained or supported by the parallel development of different models of disability – the charity, medical and social models.

Originally, decisions by substitutes were always made according to paternalistic best interests principles; but then substituted judgement and 'will and preference' tests developed to recognise the adult's own autonomous wishes and value system. However, even under the substituted judgement and 'will and preference' models, decision-making impinges on the adult's autonomy in that a substitute can be appointed without an adult's consent and even in the face of their objection. Autonomy is compromised because such an appointment is seen as intervention by the State in the adult's personal sphere.[137] In addition, under contemporary legislative regimes, decisions can be made by a substitute that conflict with the adult's expressed wishes.

Concern that decision-making by substitutes still compromises an adult's autonomy and is also discriminatory on the basis of disability has led to the promotion and development of supported decision-making. Through supported decision-making, a friend, family member or other support person can assist an adult to make 'autonomous' decisions. It aims to promote the autonomy and equality of the adult's decision-making, so as to recognise their legal capacity 'on an equal basis with others'. Supported decision-making is said to be more respectful of autonomy when it is informal, unregulated and 'least restrictive'.[138] However, there is also a drive to legislate for supported decision-making and to regulate support relationships to promote transparency and accountability.[139]

There are ongoing challenges with supported decision-making. It fails to fully account for severe and profound cognitive disability[140] and for cases when an adult decides to harm themselves or others.[141] Challenges also arise by the fineness of the line between supporting an adult's will and preferences and substituting for them.[142]

[137] See Chapter 4, Section 2.3 for an understanding of autonomy as demanding non-intrusion in the private sphere.

[138] Dinerstein (n 33) 10.

[139] *General Comment No 1* [29(d)].

[140] Wayne Martin et al, *Three Jurisdictions Report: Towards Compliance with CRPD Article 12 in Capacity/Incapacity Legislation across the UK* (Essex Autonomy Project, 2016) 34; 'this is where the General Comment comes closest to massaging the facts and closing its eyes to the actual real-life situation that many people with severe psychosocial disabilities find themselves in': David Bilchitz, 'Dignity, Fundamental Rights and Legal Capacity: Moving beyond the Paradigm Set by the General Comment on Article 12 of the Convention on the Rights of Persons with Disabilities' (2016) 32(3) *South African Journal on Human Rights* 410, 417.

[141] Arstein-Kerslake and Flynn (n 3) 482; Amita Dhanda, 'Conversations between the Proponents of the New Paradigm of Legal Capacity' (2017) 13(1) *International Journal of Law in Context* 87, 89; Craigie et al (n 2) 161; Series (n 97) 81.

[142] Carney et al (n 129) 169.

Supported decision-making under Article 12(3) CRPD is said to have been driven by the social model of disability, whereby disability is located in society and not the individual; hence, all adults can achieve decision-making autonomy if they have the appropriate social and environmental supports. Under the strict version of the social model, all adults are to be considered and treated the same, once these supports have been provided. In this way, adults with cognitive disability can have their legal capacity recognised 'on an equal basis with others'.

However, there has also been widespread critiques of the strict social model, and I describe how the interpretation of Article 12 argued for in this book is best explained by a social model that recognises the residual impacts of impairment. A strict social model aligns with a trajectory of policy development from paternalism to autonomy, and from discrimination to equality (the latter aligned with a journey from difference to sameness). Recognition of the residual impacts of impairment disrupts the simplicity of this narrative. It challenges the privileging of autonomy in human rights law and theory as a basis for personhood and demands that we seek other values, such as dignity, on which to found personhood. Recognition of the residual impacts of impairment also demands interrogation of what is meant by equality and non-discrimination in a world where we are all situated differently. Investigation of these issues forms the subject matter of Chapters 4–6.

The historical overview provided above serves as important policy and legal context for understanding the CRPD and the ongoing arguments around the interpretation of Article 12. The next section provides an overview of the main interpretive arguments found in the literature.

4 THE CRPD AND ARTICLE 12 AS A 'PARADIGM SHIFT' BUT ALSO A SITE OF CONTEST

Chapter 1 described how the CRPD grew out of an increasing recognition of the historical exclusion of people with disability from human rights agendas;[143] how the drafting process included a wide range of civil society organisations;[144] how it was agreed that the CRPD should include a broad range of human rights;[145] and that Article 12 was said to represent a 'paradigm shift' in recognising people with disability as subjects of human rights.[146]

The travaux préparatoires reveal that the contention over whether or not Article 12 should require the abolition of decision-making by substitutes was left unresolved during drafting, with the IDC ultimately acknowledging that it could accept the final text because it 'has created silence on this issue by neither prohibiting nor

[143] Section 3.
[144] Section 3.2.
[145] Section 3.3.
[146] Section 3.4.

endorsing substitutive [sic] decision making'.[147] The literature survey below shows that because of this silence, many of the arguments ventilated during drafting continued after adoption and ratification, as different commentators advocated for conflicting interpretations.

Subsection 4.1 summarises the major arguments made in favour of interpreting Article 12 as requiring the abolition of decision-making by substitutes. This is followed in Subsection 4.2 by arguments interpreting Article 12 as allowing for its retention as a last resort. This survey is intended as context for the more extensive interpretive arguments relating specifically to autonomy, dignity and equality, which are elaborated further in Chapters 4, 5 and 6, respectively.

4.1 Arguments That Recognition of Legal Capacity Demands the Abolition of Substitute Decision-Making

Throughout drafting negotiations, the IDC argued that Article 12 must require the abolition of all forms of guardianship, substitute decision-making and involuntary treatment or the paradigm shift required by the CRPD would not be achieved.[148] Following ratification of the CRPD, *General Comment No 1*, published in 2014, adopted an interpretation effectively supporting the IDC's view, affirming that adults with cognitive disability have their legal capacity recognised, their will and preferences given effect to, and their autonomy preserved. It further provided that Article 12 should be interpreted as requiring the abolition of guardianship and other substituted decision-making practices.[149] Many commentators, both before and after *General Comment No 1*'s publication, have endorsed this interpretation.

[147] Ad Hoc Committee, *Daily Summary of the Seventh Session: Discussions Related to Article 12 Equal Recognition as a Person before the Law (continued)* (18 January 2006) www.un.org/esa/socdev/enable/rights/ahc7sum18jan.htm Ad Hoc Committee ('*Daily Summary 18 January 2006*').

[148] 'Substituted decision making is based on the premise of incompetence and must not be legitimized. Supported decision making is based on the premise of competence. The two cannot exist together and successfully achieve the paradigm shift desired'. Ad Hoc Committee, *Daily Summary 18 January 2006* (n 147).

[149] 'Substitute decision making' was originally defined in *General Comment No 1* as a system where: '1) legal capacity is removed even for a single decision; 2) a substitute is appointed by someone other than the adult, potentially against their will, *and* [emphasis added] 3) a decision is made by the substitute under best interest principles'. This conjunctive definition appeared to allow for some systems of what would usually be considered substitute decision-making – i.e. in cases where *not all* of the three criteria were met. A technical reading would, e.g., allow a scenario where a substitute makes a decision based on the adult's will and preferences and not according to best interests. However, the UN Disability Committee subsequently released a corrigendum, changing the 'and' to 'or' so that if even one only of the three criteria applied, the decision-making arrangement would fall foul of Article 12. Confusingly, the status of the corrigendum is unclear: Martin et al, *Three Jurisdictions Report* (n 140) 65; Ward, Adrian, 'Abolition of Guardianship? "Best Interests" versus "Best Interpretation"' (2015) 32 *Scots Law Times* 150, 153.

4.1.1 Legal Capacity Includes the Capacity to Act and Is a Precondition to Exercising Other Rights

In arguing that Article 12 should require the abolition of all forms of decision-making by substitutes, the IDC maintained that 'legal capacity' includes both the capacity to hold rights and the capacity to act on them because 'Legal capacity is only a shell in the absence of the capacity to act'.[150] It reiterated strongly that it did not want to 'legitimise guardianship', which it described as not allowing for the provision of support.[151] Together with Inclusion International, the IDC referred to the historical abuses of guardianship, and stated that 'The time for appointment of personal representatives, even as a last resort, is over'.[152]

In 2008, a group of 32 academics, lawyers and disability advocates signed a 'Legal Opinion on Article 12 CRPD' ('the Legal Opinion') reiterating the IDC's view that legal capacity includes the capacity to act.[153] Minkowitz, as Chair of the World Network of Users and Survivors of Psychiatry and one of the signatories of the Legal Opinion, has been and continues to be a strong opponent of an interpretation of Article 12 that allows for any decision-making by substitutes, writing that: 'Without full legal capacity, a significant proportion of people with disabilities would remain outside the protection of the Convention and be unable to exercise their human rights'.[154] Her view is that the right to 'physical and mental integrity' protected by Article 17 CRPD[155] and the right to consent to (or refuse) health care protected by Article 25(d)[156] cannot be upheld unless adults have a right to active legal capacity under Article 12.[157] Morrissey similarly espouses more broadly that:

[150] Ad Hoc Committee, *Daily Summary 18 January 2006* (n 147). The term 'legal capacity' had already been used in the *CEDAW* Article 14 in requiring women's legal capacity to be recognised on the same basis as men's, particularly in relation to their ability to hold property.

[151] 'Substituted decision making is based on the premise of incompetence and must not be legitimized. Supported decision making is based on the premise of competence. The two cannot exist together and successfully achieve the paradigm shift desired'. Ad Hoc Committee, *Daily Summary 18 January 2006* (n 147).

[152] Ibid.

[153] International Disability Alliance (n 2) 1–3.

[154] Tina Minkowitz, 'Legal Capacity: Fundamental to the Rights of Persons with Disabilities' (2007) 56(1) *International Rehabilitation Review* 25, 26.

[155] Article 17 'Protecting the integrity of the person – Every person with disabilities has a right to respect for his or her physical and mental integrity on an equal basis with others'.

[156] Article 25(d) – 'Require health professionals to provide care of the same quality to persons with disabilities as to others, including on the basis of free and informed consent by, inter alia, raising awareness of the human rights, dignity, autonomy and needs of persons with disabilities …'

[157] Minkowitz, 'Legal Capacity' (n 154) 25–6; see also World Network of Users and Survivors of Psychiatry, 'Implementation Manual for the United Nations Convention on the Rights of Persons with Disabilities' (February 2008) 3, 5, 16.

> Article 12 is at the core of the CRPD and the restoration of other rights such as participation and accessibility can only become fully effective if they are underpinned by the autonomous decisions of the individual.[158]

The UN Disability Committee has similarly asserted the view that fulfilment of Article 12 rights to legal capacity are foundational to claiming and fulfilling all other rights under the CRPD, and this is a view that has been expressed by other proponents of universal legal capacity.[159]

4.1.2 Denying Legal Capacity Equates to 'Civil Death'

The IDC further maintained during negotiations that denial of legal capacity is automatically rights-denying and, like many others, explains guardianship as always resulting in 'civil death'.[160] The term 'civil death' was used to describe the status of women who upon marrying had in the past been prohibited from holding property in their own name.[161] 'Civil death' references externally the denial of legal personality associated with the appointment of a plenary guardian, together with denial of political participation rights. Arstein-Kerslake and Flynn express the view that recognition of legal capacity is required to recognise legal personhood and that denial of capacity even for a single decision cannot therefore be permitted.[162] Bach equates even partial denial of legal capacity with total social and political exclusion for adults with cognitive disability.[163] Research has found that a finding of incapacity or a label of incompetence can in itself be stigmatising

[158] Fiona Morrissey, 'The United Nations Convention on the Rights of Persons with Disabilities: A New Approach to Decision-Making in Mental Health Law' (2012) 19 *European Journal of Health Law* 423, 426.

[159] See, e.g., International Disability Alliance (n 2).

[160] Ad Hoc Committee, Daily Summary of the Third Session: Discussions Related to Article 9 Equal Recognition as a Person before the Law (26 May 2004) UN Enable – Daily summary at the Third Session of the Ad Hoc Committee – Article 9, EQUAL RECOGNITION AS A PERSON BEFORE THE LAW; Quinn and Arstein-Kerslake (n 19) 42; Theresia Degener, 'Editor's Foreword' (2017) 13(1) *International Journal of Law in Context* 1; Anna Lawson, 'The United Nations Convention on the Rights of Persons with Disabilities: New Era or False Dawn?' (2007) 34(2) *Syracuse International Journal of Law and Commerce* 563, 569; Gerard Quinn and Abigail Rekas-Rosalbo, 'Civil Death: Rethinking the Foundations of Legal Personhood for Persons with a Disability' (2016) 56 *Irish Jurist* 286, 286–9; George Szmukler, '"Capacity", "Best Interests", "Will and Preferences" and the UN Convention on the Rights of Persons with Disabilities' (2019) 18(1) *World Psychiatry: Official Journal of the World Psychiatric Association (WPA)* 34, 35; Morrissey (n 158) 427.

[161] Bernadette McSherry, 'Legal Capacity under the Convention on the Rights of Persons with Disabilities' (2012) 20 *Journal of Law and Medicine* 22, 23.

[162] Eilionóir Flynn and Anna Arstein-Kerslake, 'The Support Model of Legal Capacity: Fact, Fiction, or Fantasy?' (2014) 32(1) *Berkeley Journal of International Law* 124, 126.

[163] Michael Bach, 'Inclusive Citizenship: Refusing the Construction of "Cognitive Foreigners" in Neo-liberal Times' (2017) 4(1) *Research and Practice in Intellectual and Developmental Disabilities* 4, 10–14.

and lead to learned helplessness.[164] Philosophically, the concept of civil death is premised on acceptance of the liberal tradition whereby civil and moral personhood are denied to an individual who lacks a sufficient amount of reason to be autonomous – that is, children and adults with cognitive disability.

4.1.3 Bach and Kerzner's Proposal for 'Facilitated Decision-Making'

In 2010, Bach and Kerzner argued that Article 12 required the abolition of guardianship and decision-making by substitutes and prepared a report for the Law Reform Commission of Ontario proposing a three-tier system of supported decision-making. They proposed three different decision-making 'statuses' differentiated *not* by assessing legal capacity, but by determining differential decision-making capability. Under the 'legally independent status', an adult makes a decision without formal support and the decision is recognised as his or her own. Under 'supported decision-making status', an appointed or recognised person (a supporter) supports the adult to make a decision that accords with the adult's own will and preferences. The third status of 'facilitated decision-making' would be available to adults with 'significant disabilities' who also find themselves in situations of isolation and who have no advance planning documents.[165]

Facilitated decision-making status would apply in two circumstances. First, it would apply where an adult's will and preferences cannot be communicated; and second, when adults would otherwise have supported decision-making status except that they intend to make a decision with 'serious adverse effects'.[166] In the first situation, the facilitator must gather whatever information may be available about the person's history and circumstances in order to make a decision consistent with the adult's coherent life narrative.[167] In the second situation, the facilitator should consider which decision would best outweigh the risk of harm to the adult or improve their quality of life.

Despite Bach and Kerzner describing their model as supported decision-making, several commentators have pointed out that the facilitated decision-making status equates to substituted decision-making under a different guise.[168] The proposal has also been criticised for applying what amounts to a best interests test to protect the adult from harm. As such, it is considered by some advocates of universal legal capacity to be in breach of Article 12.[169]

[164] Bruce Winick, 'The Side Effects of Incompetency Labelling and the Implications for Mental Health Law' (1995) 1(1) *Public Policy and the Law* 6, 6.

[165] Michael Bach and Lana Kerzner, *A New Paradigm for Protecting Autonomy and the Right to Legal Capacity: Advancing Substantive Equality for Persons with Disabilities through Law, Policy and Practice* (Law Commission of Ontario, October 2010) 83–91.

[166] Ibid 92; see also Chapter 6, Section 4.1.1.

[167] Ibid 66; see also Chapter 4, Section 3.3 for an explanation of how the idea of a 'coherent life narrative' underpins some accounts of autonomy.

[168] Devi (n 2) 803; see also: Terry Carney, 'Supported Decision-Making for People with Cognitive Impairments: An Australian Perspective?' (2015) 4 *Laws* 37, 45; Parker (n 5) 387.

[169] Devi (n 2) 803; Series also describes it as a 'modified outcome' approach: Series (n 97) 87.

4.1.4 The 'Best Interpretation Principle': In Pursuit of Autonomy

General Comment No 1 provides that where it is 'not practicable to determine the will and preferences of an individual' the support person must make a 'best interpretation' of the adult's will and preferences ('the best interpretation principle'). In these cases, the adult still retains their legal capacity and the adult (not the support person) would be held legally accountable for the decision.[170]

De Bhailís and Flynn describe the best interpretation principle as reflecting:

> the presumption that it is almost always possible to come to some level of understanding of a person's values, beliefs and views, and that the person's values, beliefs and views ultimately underpin their will and preferences'.[171]

This echoes the earlier commentary of Bach and Kerzner, who also argue for universal legal capacity and refer to the importance of decisions being made that reflect the 'narrative coherence' of an adult's life so that the decision is in some cases admittedly an 'ascription by others of [the adult's] will and/or intentions'.[172] Alston describes the best interpretation principle as a legal fiction but, despite this, suggests it is 'probably' justified on the basis that it is common for legal fictions to be used 'in pursuit of important societal aims'.[173] Glen argues that there is a real value to *General Comment No 1*'s 'insistence' on will and preferences, despite its challenges, because the 'expressive' value of the law in asserting universal legal capacity can lead to cultural change in ascribing personhood to people with disability.[174] The above commentary elevates the best interpretation principle and the principle of autonomy to a privileged position, with autonomy conceived of as self-determination, the freedom to make choices and to act in accordance with one's own values.[175]

4.1.5 Substitute Decision-Making Is Discriminatory on the Basis of Disability

Despite the rhetoric of universality, *General Comment No 1* does give weight and meaning to the equality proviso – that legal capacity be recognised 'on an equal basis with others'. It explains that Article 12 allows for denials of legal capacity, provided they do not discriminate on the grounds of disability or mental incapacity.[176] *General Comment No 1* distinguishes mental capacity as 'the decision making skills of a person' from legal capacity, being 'the ability to hold rights and duties (legal standing) and to exercise those rights and duties (legal agency)'.[177] According to this

[170] *General Comment No 1* [21].
[171] de Bhailís and Flynn (n 87) 16.
[172] Bach and Kerzner (n 165) 66; see further discussion Chapter 4, Section 4.1.
[173] Bruce Alston, 'Towards Supported Decision-making: Article 12 of the Convention on the Rights of Persons with Disabilities and Guardianship Law Reform' (2017) 35(2) *Law in Context* 21, 38.
[174] Glen, 'Introducing a New Human Right' (n 2) 20.
[175] See further Chapter 4 for different meanings attributed to 'autonomy'.
[176] *General Comment No 1* [25].
[177] Ibid [13].

logic, denial of legal capacity (as the ability to hold and exercise rights) should not be denied to an adult because the adult lacks decision-making skills. It explains that on this basis, not only status and outcome tests for capacity are discriminatory but also functional tests because they test an individual's mental capacity.[178]

4.1.6 Summary and Conclusions: Article 12 Requires the Abolition of Decision-Making by Substitutes

The drafting of Article 12 was contentious from the start, with arguments centring on whether it should abolish decision-making by substitutes or allow it to be retained as a last resort. While many commentators have resorted to the travaux préparatoires to settle the interpretive position, the polarity in views expressed means that they are far from determinative. I propose that the only consensus between delegates was that the text remain silent on the issue and that there was agreement to disagree.[179]

Delegates ultimately did agree that Article 12 should include a right to legal capacity, given that the language of 'legal capacity' had been used in CEDAW. However, the IDC and Inclusion International argued that legal capacity necessarily includes not just legal standing or the capacity to hold rights, but also the right to act on and exercise those rights. For the IDC,[180] Inclusion International,[181] Minkowitz[182] and others,[183] because legal capacity means the ability to hold *and* act on rights, Article 12 prohibits any form of guardianship or decision-making by substitutes. Arguments were also made, and continue to be made, that all forms of guardianship, even the appointment of a substitute for a single decision, amounts to an adult's civil death, whereby they are denied recognition of personhood. This argument, in turn, relies on an understanding of the right to legal capacity as being a foundational right that, if unfulfilled, leaves an adult unable to claim any other rights under the CRPD.

In 2010, Bach and Kerzner developed a supported decision-making model that they proposed was compliant with Article 12 because it ostensibly abolished substitute decision-making. However, their proposal for a 'facilitated' decision-making status has been criticised as de facto substitute decision-making, with the test of 'serious adverse effects' also approximating a 'best interests' determination. The criticisms of this proposed model illustrate the challenges of recognising legal capacity and autonomy as universal. This is, as also reflected in *General Comment No 1*'s acknowledgement, that in some cases it may 'not [be] practicable to determine the

[178] Ibid [29(i)].

[179] To interpret Article 12 by referring to the travaux préparatoires to 'avoid leaving a gaping hole in the Convention … would enforce an interpretation where the negotiators could not reach a consensus'. Bartlett (n 51) 756.

[180] Ad Hoc Committee, *Daily Summary 18 January 2006* (n 147).

[181] Ibid.

[182] Minkowitz, Legal Capacity' (n 154) 25–6; see also World Network of Users and Survivors of Psychiatry (n 157) 26.

[183] International Disability Alliance (n 2) 1–3.

will and preferences of an individual'. Despite these challenges, *General Comment No 1* provides that, a 'best interpretation' of an adult's 'will and preferences' must be made and attributed to the adult as his or her own decision.

Underlying the above interpretive arguments is the view that recognition of active legal capacity amounts to recognition of autonomy; that autonomy is a human rights value to be privileged above all other values; and that legal capacity is to be privileged as a human right above all other rights. According to the logic of these arguments, any denial or limitation on legal capacity (i.e. autonomy) on the grounds of disability constitutes unlawful discrimination. *General Comment No 1* affirms this by providing expressly that 'Freedom from discrimination in the recognition of legal capacity restores autonomy'.[184] *General Comment No 1* and the literature that endorses its position argue that denial of legal capacity on the basis of mental incapacity violates Article 12 because it constitutes prohibited discrimination based on disability.

4.2 Arguments That Recognition of Legal Capacity Allows for Retention of Decision-Making by Substitutes as a Last Resort

General Comment No 1 is not legally binding but as explained in Chapter 1, it has significant influence and normative force[185] so that States Parties are not simply free to ignore General Comments without at least engaging with them.[186] Arguments in the literature for an interpretation of Article 12 as allowing for decision-making by substitutes include those directly rebutting the arguments summarised above. An overarching argument is based on acknowledgement of the hard reality that some adults do not have the autonomy to make their own decisions, and that decision-making by substitutes is therefore not discriminatory; it is not a choice, but an inevitability.

4.2.1 The Right to Legal Capacity as a Presumption that Is Rebuttable

Interpretive arguments have been made that, given the prevalence of substitute decision-making as an 'instrument of legal protection' across the world, explicit wording would be required to mandate its abolition. It would follow, therefore, that in the absence of such explicit wording in the CRPD, substitute decision-making is permitted.[187] These arguments align with notes in the drafting documents that

[184] *General Comment No 1* [33].

[185] Chapter 1, Section 3.

[186] Martin et al, *Three Jurisdictions Report*' (n 140) 56. See also CPRD Articles 35 and 36 establish a system whereby States report regularly to the UN Disability Committee on their compliance with the convention.

[187] Volker Lipp and Julian O Winn, 'Guardianship and Autonomy: Foes or Friends?' (2011) 5 *Journal of International Aging, Law & Policy* 41, 48; Martin et al, *Achieving CRPD Compliance*

provided that in 'exceptional circumstances' decisions would be allowed to protect persons with disability who could not exercise their legal capacity.[188] During drafting negotiations, Australia had argued similarly that since guardianship regimes would 'remain for the foreseeable future', the 'paradigm shift' required by Article 12 from a medical to a social model of disability could be effected by having supported decision-making as the default model but allowing for substitute decision-making in outlying cases.[189]

4.2.2 Legal Capacity Means Only the Capacity to Hold Rights

An argument for a narrow definition of legal capacity as the capacity to hold rights but not to necessarily act on them was first raised during drafting. The Coordinator suggested that: 'His impression of the group's consensus is that there will be exceptions to exercising legal capacity, but not to having legal capacity', pointing out that even adults with serious disability can inherit, own, buy and sell property, but it would only be to perform the actions of buying and selling on their behalf that a guardian would be required.[190]

There had been an attempt to emphasise this distinction by the insertion of a footnote in the draft to the effect that 'In Arabic, Chinese and Russian, the term "legal capacity" refers to "legal capacity for rights", rather than "legal capacity to act"'.[191] The implication was that all language versions would attribute the same, more limited meaning to legal capacity, thereby recognising an adult's legal capacity even when a decision was made by a substitute. While the footnote was ultimately omitted, two letters were later tabled in the UN General Assembly, the signatories representing over 40 States Parties, each letter affirming that the omission of the footnote was only a technical, not substantive, change and that legal capacity therefore attracted the more limited meaning.[192] In the context of the arguments made

(n 4) 12; McSherry argues that article 12 provides for a *presumption* of legal capacity, but acknowledges uncertainty as to when such a presumption may be displaced: McSherry (n 161) 23; Kayess and Fogarty describe Article 12 as requiring supported decision-making, but as allowing for substitute decision-making 'in some rare circumstances, after all other support measures have been exhausted': Rosemary Kayess and Ben Fogarty, 'The Rights and Dignity of Persons with Disabilities' (2007) 32(1) *Alternative Law Journal* 22, 25.

[188] *Report of the Working Group to the Ad Hoc Committee, Annex 1, 'Draft Articles for a Comprehensive and Integral International Convention on the Protection and Promotion of the Rights and Dignity of Persons with Disabilities, Draft Article 9: Equal Recognition as a Person before the Law'* A/AC.265/2004/WG/1 (14 January 2004).

[189] Ad Hoc Committee, *Daily Summary 18 January 2006* (n 147).

[190] Ad Hoc Committee, *Daily Summary of the Fifth Session* (25 January 2005) www.un.org/esa/socdev/enable/rights/ahc5sum25jan.htm ('*Daily Summary 25 January 2005*').

[191] Ad Hoc Committee, *Interim Report – Article 12 Equal Recognition as a Person before the Law*, 8th sess, A/AC.265/2006/4 (14–25 August 2006). www.un.org/esa/socdev/enable/rights/ahc8intreporte.htm

[192] *Letter Dated 5 December 2006 from the Permanent Representative of Iraq to the United Nations Addressed to the Chairman of the General Assembly*, resumed 8th sess, UN Doc

during drafting, the effect of these letters is that they endorse an interpretation of Article 12 that allows for decision-making by substitutes as a last resort.

4.2.3 Substitute Decision-Making Is Allowed Because It Is Equivalent to 100% Support

The Chair of the Ad Hoc Committee suggested that decision-making by substitutes could be allowed under a supported decision-making paradigm because it could be justified as '100% support'.[193] The IDC emphatically rejected this suggestion, but this approach has nevertheless been adopted in the ALRC Report referred to above. The ALRC argues that the paradigm shift proposed by Article 12 can be effected by replacing an inquiry into an adult's capacity with an investigation of the supports needed by them to make a decision. It goes on to recommend a set of decision-making principles to guide law reform so that: 'supported decision-making is encouraged'; and 'representative' (i.e. substitute) decision-makers are appointed only 'as a last resort'. The ALRC describes how different levels of support may be required, including full support in the form of decision-making by a representative.[194] It argues that even in liberal philosophy, autonomy is not always privileged if in doing so, harm would result to the adult or to others.[195] It recommends that where it is not possible to determine an adult's will and preferences, or where there may be risk of harm, a decision-maker should 'consider the person's human right relevant to the situation and… act in a manner promoting the personal, social, financial and cultural wellbeing of the person'.[196] However, it is yet to be seen how this would operate in practice, and Alston queries whether the test of well-being would be very different in practice from a best interests determination.[197]

The ALRC's recommendation follows the wording of Australia's interpretive declaration, which provides that Article 12 allows '…full supported or substituted decision-making arrangements, which provide for decisions to be made on behalf of a person, only where such arrangements are necessary, as a last resort and subject to safeguards'.[198] Commentators arguing for retention of decision-making by substitutes widely agree that Article 12 demands that appointments be time- and domain-limited, subject to review and other safeguards, and that denial of legal capacity under such regimes does not amount to civil death.[199]

A/AC.265/2006/5 (5 December 2006); *Letter Dated 5 December 2006 from the Permanent Representative of Finland to the United Nations Addressed to the Chairman General Assembly*, resumed 8th sess, UN Doc A/AC.265/2006/6 (5 December 2006).

[193] Ad Hoc Committee, *Daily Summary 18 January 2006* (n 147).

[194] ALRC (n 33) 94.

[195] Ibid 83–4.

[196] Ibid 15.

[197] Alston (n 173) 38.

[198] CRPD Article 4.

[199] Donnelly, *Healthcare Decision-Making and the Law* (n 76) 93.

I note that the UN Disability Committee in 2019 did to an extent endorse the ALRC Report in that it recommended that Australia, 'Implement a nationally consistent supported decision-making framework, as recommended …' by the ALRC Report. However in the same document the Committee went on to express concern about Australia's '…lack of progress made to abolish the guardianship system and substituted decision-making regime, particularly in decisions concerning forced psychiatric treatment…'.[200] It would appear that the Committee's endorsement of the ALRC's recommendations is somewhat limited, and does not extend to the ALRC's position on representative decision-making, or to its recommendation to retain the concept of capacity in the context of the law of contract.[201]

4.2.4 Substitute Decision-Making as an Article 12(4) Safeguard

Paragraph (4) of Article 12 provides that:

> States Parties shall ensure that all measures that relate to the exercise of legal capacity provide for appropriate and effective safeguards to prevent abuse in accordance with international human rights law. Such safeguards shall ensure that measures relating to the exercise of legal capacity respect the rights, will and preferences of the person, are free of conflict of interest and undue influence, are proportional and tailored to the person's circumstances, apply for the shortest time possible and are subject to regular review by a competent, independent and impartial authority or judicial body. The safeguards shall be proportional to the degree to which such measures affect the person's rights and interests.

The ALRC Report recommends that when making a decision a representative should consider the adult's 'will, preferences and rights', thereby using the language found in paragraph (4) of Article 12. The travaux préparatoires reveal that some delegates intended the above paragraph (4) to authorise substitute decision-making as a protection from potential abuse from 'supporters'.[202] This is consistent with Donnelly's view that Article 12 allows for retention of guardianship where it is personally tailored, respects will and preferences, is time-limited and subject to review.[203] Alston notes the ALRC's addition of 'rights' to the usual rubric of will and preferences, and endorses this approach to the extent that it is a 'defensible interpretation of what the treaty demands'.[204]

In recommending that a representative (or substitute decision-maker) should consider the adult's *rights* when making a decision, the ALRC is advancing its view that guardianship and decision-making by substitutes is not always rights-denying. The

[200] Committee on the Rights of Persons with Disabilities, *Concluding Observations on the Combined Second and Third Period Reports of Australia*, UN Doc CRPD/C/AUS/CO/ ('*Concluding Observations on Australia*') 6.

[201] ALRC (n 33) 290–1.

[202] Ad Hoc Committee, *Daily Summary 25 January 2005* (n 190).

[203] Donnelly, *Healthcare Decision-Making and the Law* (n 76) 93.

[204] Alston (n 173) 37–8.

ALRC's clear view is that abuse does not result from the appointment of a representative in itself but depends on the nature of the powers exercisable and exercised.[205] Several other commentators make the argument that in certain circumstances a substitute decision-maker can actually make decisions that uphold the human rights of the adult,[206] an argument that is expressly made in this book through applying the principle of indivisibility of human rights to interpret Article 12.

4.2.5 Best Interpretation Principle Could Justify a Decision Illegitimately

Interpretations of Article 12 as allowing for the retention of substitute decision-making as a last resort are frequently critical of the best interpretation principle. First, there is a lack of clarity over what 'will' and 'preference' mean, with suggestions that a person's long-term will may conflict with a more fleeting preference.[207] Second, the best interpretation principle doesn't account for the interactive process of supported decision-making.[208] In some cases, a supporter may explain real options, but in other cases an adult may merely be given a binary choice, or no choice at all.[209] Also, a supporter who spends more time with the adult may actually have the effect of determining an adult's preference.[210] Third, there is a questioning of the principle's premise that the supporter may be better placed than the adult to determine the adult's own will and preferences.[211] A corollary of this is that the principle ignores the reality that there are some things that a supporter will not and cannot know.[212] Fourth, there is an overarching concern that the best interpretation principle could be used to justify a decision illegitimately.[213] Chapter 4 considers some of these criticisms through applying the 'best interpretation' test to the particular case of 'Alex'.

[205] ALRC (n 33) 52.

[206] Anita Smith, 'Are Guardianship Laws and Practices Consistent with Human Rights Instruments?' in A Kimberly Dayton (ed), *Comparative Perspectives on Adult Guardianship* (Carolina Academic Press, 2014) 247, 268; Martin et al, *Achieving CRPD Compliance* (n 4) 19, 27.

[207] Szmukler, '"Capacity", "Best Interests", "Will and Preferences" and the UN Convention on the Rights of Persons with Disabilities' (n 160) 39; see discussion in Chapter 4, Section 3.3 on 'hierarchical' accounts of autonomy.

[208] Paul Skowron, 'Giving Substance to "the Best Interpretation of Will and Preferences"' (2019) 62 *International Journal of Law and Psychiatry* 125.

[209] Ibid 130.

[210] Ibid 128–30.

[211] Ibid 129.

[212] Mary Donnelly, 'Best Interests in the Mental Capacity Act: Time to Say Goodbye?' (2016) 24 *Medical Law Review* 318, 327; Carney, 'Searching for Workable Alternatives to Guardianship for Vulnerable Populations?' (n 83) 117.

[213] Series and Nilsson (n 78) 371. See also the concept of 'full support' that 'some commentators claim that, even then, the disabled individual's will and preference can dictate the decision, suggesting the facilitator can have an almost preternatural ability to 'divine' the individual's wishes. This is extremely problematic … It is implausible that a third party facilitator has such capacity, and potentially disingenuous …': Jeanne Snelling and Alison Douglass, 'Legal Capacity and Supported Decision-Making' in Iris Reuvecamp and John Dawson (eds), *Mental Capacity Law in New Zealand* (Thomson Reuters, 2019) 163, 170.

4.2.6 Decision-Making by Substitutes Recognises 'Hard Reality' and Is Not Discriminatory

Several commentators who advocate for the retention of decision-making by substitutes as a last resort expressly base their argument on recognition of the factual reality and inevitability that decisions *will* be made for adults at times, especially for adults with severe and profound cognitive disability. Such writers acknowledge the hard reality that in cases of advanced dementia,[214] severe and profound intellectual disability, acquired brain injury,[215] as well as for adults suffering certain psychotic conditions, adults will lack decision-making autonomy and decisions will inevitably be made for them by others.[216]

Once this 'hard reality' is acknowledged and accepted, it follows that decision-making by substitutes is not discriminatory because it happens as a matter of fact, whenever some adults do not have the capability to make their own decisions. Galderisi's view is that decision-making processes and capability are integrally dependent on mental capacity, and people with impaired capacity will be at a greater risk of exploitation than many others.[217] In the same vein, Kim writes:

> There is a place for mental capacity assessments in determining decisional authority because there is no choice in the matter … The debate should not be over whether we sometimes should decide for others. The fact is that we inevitably do, whether we acknowledge this or not.[218]

Martin et al similarly assert that, 'The crucial point is that autonomy is intrinsically related to decision making ability' and that a person who lacks decision-making ability 'in a domain, even when support is provided, cannot accurately be described as acting autonomously in that domain'.[219] Martin et al further argue that different treatment in the form of decision-making by substitutes can actually result in substantive equality,[220] a position consistent with that argued for in Chapter 6 of this book.

[214] Nick O'Neill and Carmel Peisah, 'Beware the New Paternalisms: Capacity As a Means of Achieving the Goals of the Convention on the Rights of Persons with Disabilities' (Conference Paper, World Guardianship Congress, 15–16 October 2012) 9; Cahill (n 56) 94; Martin et al, *Three Jurisdictions Report* (n 140) 11; Anita Smith (n 206) 257.

[215] Nick O'Neill, 'From Parens Patriae to Advance Personal Planning – Guardianship in Australia 1824–2016' (Conference Paper, Australian Guardianship and Administration Council National Conference: Reflecting Will and Preference in Decision-Making, 17–18 October, 2016); including, for example, in the case of PVS: Lipp and Winn (n 187) 51.

[216] Plumb (n 49) 187, 191.

[217] Galderisi (n 5) 47–8.

[218] Craigie et al (n 2) 164.

[219] Martin et al, *Achieving CRPD Compliance* (n 4) 19.

[220] Ibid 16–19. They therefore view substitute decision-making as inevitable as a last resort – e.g. for people in comas, with advanced dementia, severe brain injury, profound learning difficulties or situations involving self-harm or suicidal tendencies: Martin et al, *Three Jurisdictions Report* (n 140) 34. Further discussed in Chapter 6, Section 4.2.

4.2.7 Summary and Conclusions: Article 12 Allows for Decision-Making by Substitutes to be Retained as a Last Resort

The interpretation of Article 12 as requiring the abolition of decision-making by substitutes has been widely criticised. Arguments were made during drafting, and have continued to be made, that Article 12 only creates a very strong presumption of legal capacity that the right to legal capacity in Article 12 is limited to the ability to hold rights so that they can be exercised by others on an adult's behalf; and also that substitute decision-making is allowed because it is equivalent to 100% or 'full' support. The argument has also been made that, given the prevalence of substitute decision-making and guardianship in jurisdictions internationally, clear and unequivocal wording would be needed should the intention of Article 12 be to abolish it.

In asserting that decision-making by a substitute or representative is not in itself rights-denying, the ALRC's position is consistent with the argument in this book; but it is the extent and nature of the representative's authority and how they exercise their power that will determine whether or not an adult's rights are either upheld or denied. The ALRC's recommendations are also premised on an understanding of paragraph (4) of Article 12 as allowing for representative decision-making as a protection against undue influence or abuse by supporters, an interpretation supported by the comments of some delegates during negotiations and shared by others since.[221]

A more concrete and fundamental argument, which is built upon in later chapters, is that, as a matter of hard fact, some adults will not have decision-making capability, no matter what supports are provided. It follows that such adults do not have decision-making autonomy in a descriptive sense and that they do not therefore have legal capacity. Related to this conclusion is a view that denial of recognition of legal capacity based on mental capacity is not, therefore, discriminatory on the basis of disability but constitutes recognition of an empirical and material difference. These interpretations, based on recognition of hard reality, align with the premises of a social model of disability that recognises the residual impacts of impairment and challenges the assumption that denial of legal capacity is automatically rights-denying.

[221] I note that the UN Disability Committee in 2019 did to an extent endorse the ALRC Report in that it recommended that Australia, 'Implement a nationally consistent supported decision-making framework, as recommended in a 2014 report of the Australian Law Reform Commission entitled *Equality, Capacity and Disability in Commonwealth Laws*'. However the Committee also expressed concern 'about the lack of progress made to abolish the guardianship system and substituted decision-making regime, particularly in decisions concerning forced psychiatric treatment…'. *Concluding Observations on Australia* (n 200) 6. The endorsement of the ALRC Report would appear to be only in respect of the ALRC's recommendation to implement a supported decision-making regime. Contrary to the prescriptions of the UN Disability Committee, the ALRC Report does also recommend the retention of the concept of 'legal capacity': ALRC Report (n 33) 290–1. One article does query whether the 2019 comments of the UN Disability Committee (above) indicate a softening of its position: Alex Ruck Keene et al, 'Mental Capacity - Why Look for a Paradigm Shift?' Medical Law Review (2023) 00, 1–18 https: // doi-org/10/1093/medlaw/fwac052

5 SUMMARY, COMMENTARY AND CONCLUSIONS

The policy and legal responses to disability and decision-making have evolved from mass institutionalisation, through guardianship to supported decision-making. Such evolution is frequently portrayed as a progression from paternalistic, autonomy-denying and discriminatory practices to ones that increasingly recognise autonomy and equality as human rights. The charity model of disability described a system where adults with cognitive disability were situated outside the Rawlsian social contract and, therefore, had no entitlement to social goods, only receiving them as an act of discretionary beneficence. A so-called medical model of disability then developed and dominated, having the detrimental impact of framing people with disability as inferior and defective, and of quarantining disability as a responsibility of and problem for the individual and not of and for society. The social model developed as a critique of the medical model to argue that it was prejudiced attitudes and ableist environments that framed impairment as disability and that impairments should be accommodated by society. Article 12 reflects the influence of the social model in its requirement that legal capacity may be achieved by adults with cognitive disability if they have the supports around them to make decisions.

The social model has been widely hailed as an extremely effective and useful advocacy tool; but it has never been fully accepted by people with disability and their supporters and has been subject to wide critique and significant review. Criticism has focused on what I refer to as the 'strict' social model, which effectively denies the reality of impairment, or the lived experience of impairment and its associated disadvantages. 'Mixed' models, referred to variously as 'biopsychosocial', 'interactive' or 'multi-factorial' have been suggested to address the limitations of the strict social model and the UN Disability Committee itself refers to a 'human rights model' of disability that recognises the impacts of impairment (albeit also endorsing recognition of universal legal capacity). I endorse what I refer to as a social model that recognises the residual impacts of impairment, arguing in later chapters that such a model demands an interpretation of Article 12 that allows for decision-making by substitutes as a last resort.

The development of the different models of disability reflects the evolution of different legal and policy responses to disability and decision-making over time, responses that have increasingly responded to calls for recognition of autonomy and personhood and the end of discrimination. Mass institutionalisation was replaced by personal guardianship, and within guardianship, capacity tests and decision-making principles evolved from paternalistic models to ones that increasingly recognised individuality and autonomy. Discriminatory capacity tests based on status and outcome have increasingly been replaced with functional tests that consider an adult's ability to understand, retain and weigh relevant information, and communicate a decision. Decision-making according to a paternalistic best interests test has started to be replaced or complemented by the substituted judgement test,

whereby a substitute makes a decision in accordance with the adult's will and preferences. Since the 1990s, supported decision-making, both informal and legislated, has begun to be practised alongside decision-making by substitutes, in increasing efforts to recognise the autonomy of adults with cognitive disability. There is wide recognition, however, that the supported decision-making model is challenged by hard cases. These hard cases include decision-making by adults with severe and profound cognitive disability, where supported decisions may result in harm and when there is a fine line as to whether the adult or the supporter has actually made the decision.

Article 12 now drives the move to supported decision-making, although with ongoing contention as to whether it allows for decision-making by substitutes as a last resort. The interpretation of Article 12 as requiring the abolition of decision-making by substitutes has been bolstered by the authoritative (although not binding) prescriptions of *General Comment No 1*. Interpretative arguments supporting the abolition of decision-making by substitutes are that legal capacity includes the capacity to act, and that denial of legal capacity amounts to civil death and loss of personhood. The privileging of the value of autonomy in these arguments is especially evident in accompanying assertions that it is always possible to elicit an adult's will and preferences or make a 'best interpretation' of them; and that denial of legal capacity on the basis of mental incapacity is always discriminatory because it amounts to a denial of autonomy.

Contrary arguments are as follows: Article 12 only creates a presumption of capacity; legal capacity means only capacity to hold rights (and not to act on them); substitute decision-making amounts to 100% support; the best interpretation principle lacks transparency and accountability; decision-making by substitutes is not always rights-denying; Article 12(4) allows for decision-making by substitutes as a safeguard. An overarching argument is based on recognition of 'hard reality', which effectively acknowledges the residual impacts of impairment. In hard cases where an adult does not, in fact, have decision-making ability, decision-making by a substitute is not in itself discriminatory. In some cases, decision-making by substitutes will be inevitable; therefore, it should be made transparent and accountable.

I argue in this book against the privileging of the right to legal capacity as a human right foundational to all others. Such a privileging is unsustainable because first, it assumes that individual rights are independent and bounded, and second, it is linked with the privileging of civil–political over socio-economic rights. The principle of indivisibility of human rights challenges these assumptions and Chapter 3 begins the investigation of why and how the principle of indivisibility drives an interpretation of Article 12 as requiring decision-making by substitutes as a last resort.